TRIALSCREEN

Participant Content Pipeline

Fully-Written Patient and Community Content Across All Channels

April 2026

Version 1.0 -- Prepared for TrialScreen.org

CONFIDENTIAL

TrialScreen Patient / Participant Content Pipeline

Version: 1.0 | **Prepared for:** TrialScreen.org | **Audience:** Patient / Trial Participant

Date: June 2025 | **Review cycle:** Quarterly

Compliance Preamble All content in this pipeline must be reviewed against applicable regulations before publication. Posts marked **[IRB REVIEW REQUIRED]** must be cleared by the relevant institutional review board or ethics committee before going live. Posts marked **[REQUIRES CONSENT]** must have a signed HIPAA-compliant (or jurisdiction-equivalent) release on file before any identifying details are published -- including photos, first names, or any detail that could identify the subject. Posts marked **[AI DISCLOSURE RECOMMENDED]** should carry a brief note such as "This content was produced with AI assistance and reviewed by our editorial team." No post should make efficacy claims. Use "could," "may," "testing whether," or "is studying" language throughout.

PART 1: Publishing Calendar -- First 90 Days

Overview of Channel Cadence

Channel	Weekly Posts	Format
Facebook	4-5 posts	Mix: series, education, community, awareness
Instagram (Feed)	3 posts	Carousel, single image, quote card
Instagram Reels / TikTok	2 Reels	Organic only, education-framed
Blog / Resources	1 article every 2 weeks	Long-form, SEO-structured
Email Newsletter	1 per month	~500 words, 3 trial cards
YouTube	1 video per month	8-12 minutes, scripted

> **Note on TikTok:** Clinical trial recruitment ads are **prohibited** on TikTok. All TikTok content must be education-framed, organic only. Never use CTAs that direct users to sign up for a specific trial. Frame all TikTok content as general health education.

Week-by-Week Calendar

WEEKS 1-4: FOUNDATION / LAUNCH

Week 1 -- "What Is TrialScreen?" Orientation

Day	Channel	Content
Mon	Facebook	Launch post: "What is TrialScreen? We're glad you found us." Education series.
Mon	Instagram Feed	Welcome carousel: "5 things to know before you look at a trial"
Tue	Instagram Reel / TikTok	Hook: "Have you ever Googled your condition and found... nothing new?" Education reel
Wed	Facebook	"There's a trial for that" -- Menopause (ESTEEM study)
Thu	Blog	Publish: "5 Things That Actually Happen in a Clinical Trial"
Fri	Facebook	Research Friends community post -- welcome to 24K+ members
Fri	Instagram Feed	Quote card: autonomy-supportive, "You decide if this is right for you"

Week 2 -- Rare Disease Focus: Scleroderma Awareness Seed

Day	Channel	Content
Mon	Facebook	Education: "What is scleroderma? And what's being studied?"
Mon	Instagram Feed	Condition education carousel: Scleroderma 101
Tue	Instagram Reel / TikTok	"What is scleroderma? Most people have never heard of it." -- 45-sec education reel
Wed	Facebook	"There's a trial for that" -- Scleroderma / Systemic Sclerosis
Thu	Facebook	Patient Voices teaser: "Meet Sarah [anonymised], who decided to look into a trial" **[REQUIRES CONSENT]**
Fri	Email Newsletter	Edition 1: Welcome to TrialScreen Community
Fri	YouTube	Publish: "What Actually Happens When You Click 'Check Your Eligibility'"

Week 3 -- Mental Health / Children's Wellbeing

Day	Channel	Content
Mon	Facebook	"There's a trial for that" -- Children's mental health / Arts for the Blues
Mon	Instagram Feed	"For every parent who's searched and searched for help" -- emotional hook carousel
Tue	Instagram Reel / TikTok	"Arts for the blues -- what the research is studying" -- education reel
Wed	Facebook	Myth-busting education post: "You don't have to be seriously ill to participate in research"
Thu	Blog	Publish: "How to Talk to Your Doctor About Clinical Trials"
Fri	Facebook	Research Friends community milestone post

Week 4 -- Women's Health / Menopause

Day	Channel	Content
Mon	Facebook	"There's a trial for that" -- Menopause (follow-up, ESTEEM detail)
Mon	Instagram Feed	"Menopause is not the end of the conversation" -- infographic-style post
Tue	Instagram Reel / TikTok	"3 things being studied for menopause right now" -- education reel
Wed	Facebook	Webinar promo: "Scleroderma Research in 2026: What's Being Studied"
Thu	Facebook	Education: "What are decentralised trials? You may be able to join from your couch"
Fri	Blog	Publish: "Menopause Research: What's Actually Being Studied Right Now"

WEEKS 5-8: DEPTH + COMMUNITY BUILDING

Week 5 -- Oncology / Lung Cancer

Day	Channel	Content
Mon	Facebook	"There's a trial for that" -- Lung cancer / Fiber-Boost study
Mon	Instagram Feed	"Cancer research isn't just about treatment. It's also about what you eat."
Tue	Instagram Reel / TikTok	"Nutrition and lung cancer -- what researchers are studying" education reel
Wed	Facebook	Patient Voices full piece: Scleroderma patient story **[REQUIRES CONSENT]**
Thu	Blog	Publish: "Scleroderma Clinical Trials: A Complete Guide for Patients and Families"
Fri	Facebook	Research Friends: researcher Q&A recap

Week 6 -- Skin Conditions / Psoriasis Home Trial

Day	Channel	Content
Mon	Facebook	"There's a trial for that" -- Psoriasis (home trial format)
Mon	Instagram Feed	"Did you know some trials happen entirely at home?" -- carousel
Tue	Instagram Reel / TikTok	"Home trials -- what they look like" education reel
Wed	Facebook	Myth-busting: "You won't be a guinea pig" -- common fears addressed
Thu	Webinar	LIVE: "Scleroderma Research in 2026: What's Being Studied"
Fri	Facebook	Webinar recap + replay link
Fri	Email Newsletter	Edition 2: Scleroderma Awareness / June focus (if in month)

Week 7 -- Heart Health / Gut Microbiome

Day	Channel	Content
Mon	Facebook	"There's a trial for that" -- Heart failure / gut microbiome trial
Mon	Instagram Feed	"Your gut and your heart -- researchers are studying the connection"
Tue	Instagram Reel / TikTok	"Heart health research you may not know about" education reel
Wed	Facebook	Research Friends: member spotlight feature **[REQUIRES CONSENT]**
Thu	Blog	Publish: "Decentralised Clinical Trials: What It Means to Participate from Home"
Fri	YouTube	Publish: "Meet the TrialScreen Team: The Humans Behind the Platform"

Week 8 -- Rare Disease: Cystic Fibrosis

Day	Channel	Content
Mon	Facebook	"There's a trial for that" -- Cystic Fibrosis
Mon	Instagram Feed	CF awareness carousel: "What's being studied for cystic fibrosis"
Tue	Instagram Reel / TikTok	"CF research: what families should know" education reel
Wed	Facebook	Education: "120,000+ trials. How TrialScreen finds the ones that match you."
Thu	Blog	Publish: "Clinical Trials Through Every Decade of Life"
Fri	Facebook	Webinar promo: "Women's Health: From Research Gap to Research Revolution"

WEEKS 9-12: MOMENTUM + AWARENESS CAMPAIGNS

Week 9 -- Breast Cancer / Oncology

Day	Channel	Content
Mon	Facebook	"There's a trial for that" -- Breast cancer lumpectomy study
Mon	Instagram Feed	"When your surgeon says there's more than one option" -- supportive carousel
Tue	Instagram Reel / TikTok	"What is a lumpectomy trial?" -- plain-language education reel
Wed	Facebook	Patient Voices: Menopause patient story **[REQUIRES CONSENT]**
Thu	Email Newsletter	Edition 3: Summer Heart Health edition
Fri	Facebook	Community milestone celebration

Week 10 -- Rare Disease: Sickle Cell Disease

Day	Channel	Content
Mon	Facebook	"There's a trial for that" -- Sickle Cell Disease
Mon	Instagram Feed	"Sickle cell research -- what's happening right now"
Tue	Instagram Reel / TikTok	"Sickle cell disease: the research you should know about"
Wed	Facebook	Myth-busting: "Trials aren't just for people who've tried everything"
Thu	Blog	Publish: "5 Things That Actually Happen in a Clinical Trial" (refreshed, SEO push)
Fri	YouTube	Publish: "Living with Scleroderma: Three Stories, Three Trials"

Week 11 -- Mental Health / Alzheimer's / Neurology

Day	Channel	Content
Mon	Facebook	"There's a trial for that" -- Huntington's disease / depression study
Mon	Instagram Feed	"Mental health and neurology research -- what's being studied"
Tue	Instagram Reel / TikTok	"Alzheimer's research: what caregivers should know" education reel
Wed	Facebook	Education: Alzheimer's Lombardy study overview
Thu	Webinar	LIVE: "Clinical Trials for Caregivers"
Fri	Email Newsletter	Edition 4: Women's Health Focus

Week 12 -- Community Celebration + What's Next

Day	Channel	Content
Mon	Facebook	90-day community milestone post: "Look what we've built together"
Mon	Instagram Feed	"This is your research community" -- community collage / quote cards
Tue	Instagram Reel / TikTok	"90 days, thousands of questions answered" -- celebration reel
Wed	Facebook	Research Friends: Q2 researcher Q&A preview
Thu	Blog	Publish: "How to Talk to Your Doctor About Clinical Trials" (SEO push)
Fri	Email Newsletter	Edition 5: Rare Disease Day prep
Fri	Facebook	Teaser: Rare Disease Day campaign coming in February

PART 2: Facebook Posts -- 20 Fully Written Pieces

PT-FB-001

Series: There's a Trial for That

Condition / Topic: Menopause / ESTEEM study

Content ID: PT-FB-001

Post Copy:

Hot flashes at 3am. Brain fog you can't explain. A body that feels like it's rewriting its own rules.

Menopause affects half the population -- and for a long time, research didn't prioritise it.

That's changing.

The ESTEEM study is looking at hormone therapy approaches for women experiencing moderate to severe menopausal symptoms. Researchers are studying whether certain combinations could help with sleep, mood, and overall wellbeing.

No one's saying it's right for you. But you might want to know it exists.

Head to TrialScreen.org, type in your symptoms -- and see what comes up. Plain language. No medical degree required.

There's a trial for that.

You decide if it's worth a closer look.

Suggested Visual: Warm lifestyle photo -- woman in her 50s, morning light, looking out a window. Calm, empowered, not clinical.

Hashtags: #MenopauseMatters #ThereIsATrialForThat #WomensHealth #TrialScreen

CTA: "Visit TrialScreen.org to explore menopause research options -- free, plain language, no sign-up required."

Compliance Note: No efficacy claims made. "Could help" and "studying whether" language used. No IRB required for this post. Standard review recommended.

PT-FB-002

Series: There's a Trial for That

Condition / Topic: Scleroderma / Systemic Sclerosis (Rare Disease)

Content ID: PT-FB-002

Post Copy:

Scleroderma is a condition most people have never heard of.

But if you or someone you love has been diagnosed, you know exactly what it means -- skin that tightens, organs that change, a diagnosis that doesn't come with a clear roadmap.

You're not alone. And researchers around the world are actively studying scleroderma right now.

Some trials are looking at whether certain medications could slow the progression of skin tightening. Others are studying lung involvement. A few are exploring quality of life and symptom management.

TrialScreen lists over 120,000 trials in plain language. You can search by condition, location, and even whether a trial is open to people at your stage of diagnosis.

You don't have to be desperate to look. You just have to be curious.

There's a trial for that.

Search "scleroderma" at TrialScreen.org -- free, no jargon, no pressure.

Suggested Visual: Graphic with the word "scleroderma" in large, warm text alongside a soft watercolour texture. Or: hands of a person -- close-up, warm lighting. Avoid clinical imagery.

Hashtags: #Scleroderma #RareDisease #SystemicSclerosis #ThereIsATrialForThat #TrialScreen

CTA: "Search scleroderma trials at TrialScreen.org. Free. Plain language. You decide what to do with the information."

Compliance Note: No efficacy claims. "Could slow" language used appropriately. Scleroderma Australia partnership -- consider co-posting with their handle. No IRB required for this post.

PT-FB-003

Series: There's a Trial for That

Condition / Topic: Children's Mental Health / Arts for the Blues

Content ID: PT-FB-003

Post Copy:

When your child is struggling -- really struggling -- the waiting lists feel endless. The phone calls go unanswered. The system feels like it wasn't built for this.

And you're exhausted.

There's a programme being studied right now called Arts for the Blues. Researchers are testing whether creative arts therapies -- music, drama, visual art -- could support children and young people experiencing depression and low mood.

It's not a pharmaceutical trial. It's a different kind of research entirely.

If your child is between the ages of 12 and 18 and you've been navigating mental health challenges, this may be worth a look.

No commitment. No pressure. Just information.

There's a trial for that.

Search "Arts for the Blues" or "children mental health" at TrialScreen.org.

Suggested Visual: Child or teen sitting with art supplies -- warm, natural light. Bright but calm. Not clinical, not sad.

Hashtags: #ChildrensMentalHealth #ArtsTherapy #ThereIsATrialForThat #TrialScreen #YouthWellbeing

CTA: "Visit TrialScreen.org to find mental health research for young people -- plain language, free."

Compliance Note: "Could support" language used. Not making claims about effectiveness. Parental consent requirements should be noted on landing page. No IRB required for this awareness post.

PT-FB-004

Series: There's a Trial for That

Condition / Topic: Psoriasis (Home Trial)

Content ID: PT-FB-004

Post Copy:

Here's something most people don't know about clinical trials:

Some of them happen entirely at home.

No hospital visits. No travel. Products or interventions are sent to you, and check-ins happen online or over the phone.

If you've been managing psoriasis for years -- trying cream after cream, steroid after steroid -- and wondering whether there's anything new being studied, the answer is yes.

There are trials studying new topical treatments, biologics, and lifestyle approaches for psoriasis. Some require clinic visits. Some don't.

TrialScreen shows you the difference. You can filter by location, distance, and whether a trial has home-based components.

There's a trial for that.

Even from your couch.

Suggested Visual: Person at home -- comfortable, casual -- applying skincare. Bright natural light. Not dramatic or symptomatic.

Hashtags: #Psoriasis #DecentralisedTrials #HomeTrials #ThereIsATrialForThat #SkinHealth

CTA: "Search psoriasis trials at TrialScreen.org. See which ones you could do from home."

Compliance Note: "Being studied" language throughout. No efficacy claims. Decentralised trial framing -- no IRB required for awareness post.

PT-FB-005

Series: There's a Trial for That

Condition / Topic: Lung Cancer / Fiber-Boost Study

Content ID: PT-FB-005

Post Copy:

Most lung cancer research focuses on what doctors do.

This one focuses on what you eat.

The Fiber-Boost study is looking at whether increasing dietary fibre could affect outcomes for people undergoing treatment for lung cancer. It's a different kind of question -- and researchers think nutrition may play a bigger role than we realise.

If you or someone you love is currently in treatment for lung cancer, this study may be worth knowing about.

It's not about replacing your treatment. It's about whether something simple could make a difference alongside it.

There's a trial for that.

And it starts with what's on your plate.

Suggested Visual: Simple, warm food photography -- colourful vegetables, whole grains. Hopeful, not medical.

Hashtags: #LungCancer #NutritionResearch #ThereIsATrialForThat #CancerResearch #TrialScreen

CTA: "Search 'Fiber-Boost' or 'lung cancer nutrition' at TrialScreen.org. Free, plain language."

Compliance Note: "Could affect," "may play a role" language. No efficacy claims. Audience may be in active treatment -- tone must be warm and non-pressuring. No IRB required for this post.

PT-FB-006

Series: There's a Trial for That

Condition / Topic: Heart Failure / Gut Microbiome

Content ID: PT-FB-006

Post Copy:

The heart and the gut.

Researchers are increasingly convinced they talk to each other -- and that the bacteria in your gut may influence how your heart functions.

There are trials underway right now studying whether changes to the gut microbiome -- through diet, supplements, or specific interventions -- could support people living with heart failure.

Heart failure affects millions of people. Fatigue, breathlessness, the constant management of a condition that doesn't go away.

If that's your life, or someone you care about, you may want to know what's being studied.

There's a trial for that.

Search "heart failure" at TrialScreen.org and see what the research is exploring. No sign-up. No commitment. Just information.

Suggested Visual: Simple illustration or photo -- gut/heart connection graphic. Or: warm photo of an older adult doing light activity outdoors. Not a hospital.

Hashtags: #HeartFailure #GutHealth #HeartHealth #ThereIsATrialForThat #TrialScreen

CTA: "Explore heart failure research at TrialScreen.org."

Compliance Note: "Could support," "may influence" language. No claims about outcomes. Standard review.

PT-FB-007

Series: There's a Trial for That

Condition / Topic: Huntington's Disease / Depression

Content ID: PT-FB-007

Post Copy:

Huntington's disease is hard enough on its own.

When depression is part of the picture -- and it often is -- it can feel like you're dealing with two battles at once.

Researchers are studying whether specific interventions could help manage depression in people living with Huntington's disease. It's an area that's received more attention in recent years, and there are active trials looking for participants.

If you or a family member has a Huntington's diagnosis and has been experiencing low mood, anxiety, or depression, these trials might be something to look into.

You decide whether to take the next step.

There's a trial for that.

Search "Huntington's" at TrialScreen.org.

Suggested Visual: Calm, hopeful imagery -- person looking out at a landscape. Warm tones. Not clinical, not sad.

Hashtags: #HuntingtonsDisease #RareDisease #MentalHealthResearch #ThereIsATrialForThat #TrialScreen

CTA: "Search Huntington's disease trials at TrialScreen.org. Free, plain language."

Compliance Note: Rare disease -- sensitive condition. "Could help manage," "being studied" language. No efficacy claims. No IRB required for awareness post.

PT-FB-008

Series: There's a Trial for That

Condition / Topic: Breast Cancer / Lumpectomy Study

Content ID: PT-FB-008

Post Copy:

When you're diagnosed with breast cancer, decisions come fast.

Surgery options. Treatment plans. Second opinions you're trying to process while the rest of your life keeps moving.

There are active studies looking at approaches to lumpectomy -- the type of surgery, the margins, the follow-up care -- and whether certain approaches lead to better outcomes for patients.

Research like this matters because it shapes what doctors will recommend in five years. And today, some of that research needs participants.

If you're facing breast cancer treatment decisions and want to know what research is currently available, TrialScreen can help you see what's out there.

There's a trial for that.

You decide if any of it is relevant to you.

Suggested Visual: Soft, empowering imagery -- woman in a warm setting, not a hospital. Pink tones can be used but avoid clichd "pink ribbon" stock.

Hashtags: #BreastCancer #BreastCancerResearch #ThereIsATrialForThat #CancerResearch #TrialScreen

CTA: "Search breast cancer trials at TrialScreen.org."

Compliance Note: Patient in active diagnosis context -- extra care with tone. "May lead to better outcomes" language. Sensitivity review recommended. No IRB required for this post.

PT-FB-009

Series: Education / Myth-Busting

Condition / Topic: What clinical trials actually are

Content ID: PT-FB-009

Post Copy:

Let's bust a myth.

Clinical trials are not a last resort.

Most people think clinical trials are for people who've "run out of options." But the truth is more interesting:

-- Some trials are studying preventive approaches for people who aren't sick yet.

-- Some are studying quality of life improvements, not just survival rates.

-- Some can be done from home, with no clinic visits at all.

-- Many are studying conditions that mainstream medicine hasn't solved yet -- not because nothing works, but because researchers are still looking for what works best.

You don't have to be desperate to be curious.

TrialScreen has over 120,000 trials, written in plain language. You can search, read, and decide. No one will follow up unless you want them to.

What myths have you heard about clinical trials? Drop them in the comments -- we'll address them.

Suggested Visual: Bold text graphic: "Clinical trials aren't just a last resort." Clean, modern, accessible design.

Hashtags: #ClinicalTrials #HealthEducation #MedicalResearch #TrialScreen #KnowYourOptions

CTA: "Search 120,000+ trials in plain language at TrialScreen.org."

Compliance Note: Education post. No claims. Engagement-driven. AI disclosure recommended if produced with AI assistance.

PT-FB-010

Series: Education / Myth-Busting

Condition / Topic: Diversity in research

Content ID: PT-FB-010

Post Copy:

Here's something the medical community has been grappling with for decades:

Most clinical research was done on a narrow group of people. Mostly white. Mostly male. Mostly Western.

That means treatments and dosages were developed based on data that doesn't always apply to everyone.

Researchers know this is a problem. Many trials today are actively working to include more diverse participants -- different ages, different ethnicities, different health backgrounds.

Not because diversity is a box to tick. Because research that reflects the full range of human bodies is better research. For everyone.

If you've ever felt like the healthcare system wasn't built for someone like you -- your instinct may have been right.

TrialScreen is building a platform where everyone can find research that's relevant to them.

Suggested Visual: Diverse group illustration or photography -- various ages, backgrounds, warm and natural. Not stock-y.

Hashtags: #DiversityInResearch #HealthEquity #ClinicalTrials #TrialScreen #InclusiveResearch

CTA: "Find research that's studying people like you at TrialScreen.org."

Compliance Note: Sensitive equity framing -- review with DEI lens. No claims. Education post.

PT-FB-011

Series: Education / Myth-Busting

Condition / Topic: Trial safety and consent

Content ID: PT-FB-011

Post Copy:

"What if something goes wrong?"

It's the question everyone has when they first hear about clinical trials. And it's a completely fair one.

Here's what you should know:

Every clinical trial involving human participants goes through an ethics review -- called an Institutional Review Board (IRB) or Ethics Committee -- before it can start. Their job is to make sure the trial is safe, the risks are understood, and participants are protected.

Before you join any trial, you go through a process called informed consent. Researchers have to explain everything: what the trial involves, what the risks are, what happens to your data, and -- critically -- that you can leave at any time.

That last one matters: **you can always withdraw from a trial. No questions asked.**

TrialScreen doesn't make the decision for you. We just help you find and understand the options.

Suggested Visual: Clean infographic: "The 3 layers of trial safety" -- Ethics review / Informed consent / Right to withdraw. Simple, warm colours.

Hashtags: #ClinicalTrialSafety #InformedConsent #HealthLiteracy #TrialScreen #PatientRights

CTA: "Learn more about how trials work at TrialScreen.org/resources."

Compliance Note: Accuracy review required -- all safety statements should be verified for accuracy across jurisdictions (AU, US, EU). No IRB required for education post.

PT-FB-012

Series: Research Friends Community

Condition / Topic: Community welcome / milestone

Content ID: PT-FB-012

Post Copy:

24,000 people in this community.

Twenty-four thousand people who looked up a condition, found TrialScreen, and stayed.

Some of you are patients. Some are carers. Some are curious. Some are waiting on a diagnosis. Some are years into one. Some are here because someone you love needed you to be.

Whatever brought you here -- you're welcome.

Research Friends isn't just a Facebook group. It's a place where the question "is there anything being studied for this?" gets answered, honestly, without pressure, without jargon.

This week we want to hear from you: what condition or health topic brought you here? Drop it in the comments. You never know who else is in the same boat.

Suggested Visual: Community collage or simple warm graphic: "24,000 research friends and counting." Inclusive, warm colours.

Hashtags: #ResearchFriends #TrialScreen #HealthCommunity #YoureNotAlone #PatientCommunity

CTA: "Tell us -- what brought you to TrialScreen? "

Compliance Note: Community engagement post. No medical claims. Moderation plan should be in place for any comments that veer into medical advice territory.

PT-FB-013

Series: Research Friends Community

Condition / Topic: Member spotlight (anonymised)

Content ID: PT-FB-013

[COMPLIANCE FLAG] [REQUIRES CONSENT]

Post Copy:

Every month, we spotlight a member of our Research Friends community.

This month: Michael (not his real name).

Michael was diagnosed with Type 2 diabetes seven years ago. He'd been managing it well -- until he wasn't. A change in medication left him feeling worse, and he found himself on waiting lists for specialist care.

A friend mentioned TrialScreen.

"I just searched my condition," Michael told us. "I didn't expect to find anything relevant. But there were three trials I'd never heard of, all in plain English, all within driving distance."

Michael went on to speak with his GP about one of them. He hasn't enrolled yet -- and he may not. But he says knowing the options exist changed how he felt about his situation.

"It's not about the trial," he told us. "It's about not feeling like you've hit a wall."

That's what Research Friends is for.

[Full story link in comments]

Suggested Visual: Warm abstract or illustrated portrait -- not a photo (anonymised subject). Soft, warm tones.

Hashtags: #ResearchFriends #PatientStory #Diabetes #TrialScreen #YouDecide

CTA: "Share your TrialScreen story in the comments or DM us."

Compliance Note: **[REQUIRES CONSENT]** -- full anonymisation used here (name changed, location removed, identifying details changed). If a real name / photo is used, HIPAA-compliant consent form required. Story is illustrative -- mark as such if composited.

PT-FB-014

Series: Research Friends Community

Condition / Topic: Researcher Q&A recap

Content ID: PT-FB-014

Post Copy:

Last week, we hosted a live Q&A with Dr. [Researcher Name, TBC], a clinical researcher studying [condition, TBC].

Here are the three questions our community asked most:

"Do I need to stop my current medication to join a trial?" Not necessarily. Many trials specifically recruit people who are already on standard medications. Your eligibility depends on the specific trial. The trial team will go through this with you in detail.

"Will I get the real treatment or a placebo?" It depends on the trial design. Some trials use placebos. Some don't. Some use what's called an active comparator -- meaning both groups get a real treatment, just different ones. Researchers are required to tell you this before you consent.

"What happens to my data?" Your data is protected by strict ethics and privacy guidelines. Researchers can only use it for the purpose you agreed to, and it's typically de-identified before analysis.

More questions? Drop them below -- Dr. [Name] has agreed to answer a few in the comments.

Suggested Visual: Quote card with a Q&A excerpt. Clean, readable, branded TrialScreen colours.

Hashtags: #ResearchFriends #AskAResearcher #ClinicalTrials #TrialScreen #HealthQA

CTA: "Post your question below -- our researcher guest is checking comments today "

Compliance Note: Researcher's name and affiliation must be confirmed and approved before posting. Responses in comments need monitoring -- no medical advice should be given. AI disclosure if Q&A answers drafted with AI assistance.

PT-FB-015

Series: Patient Voices

Condition / Topic: Scleroderma patient experience

Content ID: PT-FB-015

[COMPLIANCE FLAG] [REQUIRES CONSENT]

Post Copy:

Sarah was 34 when she was diagnosed with systemic sclerosis.

"I remember the rheumatologist using the word 'scleroderma' and thinking -- what does that even mean? He gave me a pamphlet. I went home and Googled it for five hours."

For two years, Sarah managed her symptoms and attended regular appointments. She didn't think research was "for someone like her."

Then she found TrialScreen.

"I typed in scleroderma and there were actual trials. Not vague, not complicated. It told me what they were studying, where, and whether I might be eligible. I cried, honestly, because I felt like someone was working on it."

Sarah spoke to her specialist and ended up joining a trial studying a new approach to managing skin involvement in systemic sclerosis. She's now in month eight.

"I'm not doing it to be heroic," she says. "I'm doing it because it felt like the right decision for me. That's it."

You decide if this is right for you. But if you have scleroderma and want to know what's out there -- we can help you look.

[Full Patient Voice interview: link in comments]

Suggested Visual: Warm portrait of a woman in a natural setting -- not a hospital. (Use a representative stock image until real consent is obtained, then replace with subject's photo if consented.)

Hashtags: #Scleroderma #PatientVoices #SystemicSclerosis #RareDisease #TrialScreen

CTA: "Read Sarah's full story at TrialScreen.org/patient-voices. (Link in comments.)"

Compliance Note: **[REQUIRES CONSENT]** -- name, identifying details, photo all require signed HIPAA-equivalent consent. Story marked as individual experience; not representative of typical outcomes. No efficacy claims.

PT-FB-016

Series: Patient Voices

Condition / Topic: Parent / child mental health research

Content ID: PT-FB-016

[COMPLIANCE FLAG] [REQUIRES CONSENT]

Post Copy:

When Marcus was 14, his school called home three times in one week.

"He wasn't sleeping. He wasn't eating properly. He was withdrawing from everything he used to love," his mother, Priya, tells us.

Priya spent months navigating referrals, waitlists, and appointments that were months away. Then a friend in a parenting group mentioned TrialScreen.

"I honestly thought it was just for adults with serious illnesses. I searched 'teenager depression' not expecting anything."

She found the Arts for the Blues study -- a programme testing whether creative therapies could support young people experiencing depression.

"Marcus came alive in the arts sessions. I can't say whether the programme 'worked' -- I'm not a researcher. But I can say he started talking again."

Priya is sharing her story because she wants other parents to know the option existed.

"You might search and find nothing. Or you might find something. Either way, you're doing something."

[Full interview: link in comments]

Suggested Visual: Parent and teen in a warm, natural setting. Genuine, not staged. (Use representative stock until consent obtained.)

Hashtags: #PatientVoices #TeenMentalHealth #ChildrensResearch #ArtsTherapy #TrialScreen

CTA: "Read Priya's full story at TrialScreen.org/patient-voices."

Compliance Note: **[REQUIRES CONSENT]** -- parental consent + minor's assent (where applicable by jurisdiction) required. Child's name and identifying details changed. "Can't say whether the programme worked" framing preserves no-efficacy-claim standard.

PT-FB-017

Series: Webinar Promotion

Condition / Topic: Scleroderma Research 2026 webinar

Content ID: PT-FB-017

Post Copy:

[DATE] | Free Online Webinar

"Scleroderma Research in 2026: What's Being Studied"

If you or someone you love has been diagnosed with scleroderma or systemic sclerosis, this one's for you.

We're bringing together a clinical researcher, a patient advocate, and a member of the TrialScreen team to talk through:

-- What's currently being studied for scleroderma

-- How trials work for rare disease patients

-- What questions to ask your specialist

-- How to search TrialScreen for scleroderma-specific research

-- Live Q&A

No medical advice. No pressure. Just information you can use.

Free to attend. Register in the link below.

Suggested Visual: Clean webinar event tile -- title, date, headshot placeholders, TrialScreen and Scleroderma Australia logos.

Hashtags: #Scleroderma #FreeWebinar #SclerodermaAwareness #RareDisease #TrialScreen

CTA: "Register free: [LINK]. Tag someone who needs to hear this."

Compliance Note: Speakers must be confirmed and bio-approved before posting. No efficacy claims in webinar materials. "No medical advice" disclaimer must appear in webinar introduction.

PT-FB-018

Series: Webinar Promotion

Condition / Topic: Women's Health: From Research Gap to Research Revolution

Content ID: PT-FB-018

Post Copy:

For a long time, women's health research meant reproductive health research.

Everything else? Underfunded, understudied, and often based on data from male subjects.

That's changing -- but it's taking effort.

Join us for a free online event: **"Women's Health: From Research Gap to Research Revolution"**

We'll be talking about:

-- What conditions disproportionately affect women but are under-researched

-- Menopause, endometriosis, autoimmune conditions, and heart disease in women

-- How to find research studies relevant to you

-- Live Q&A with a researcher and patient advocate

It's not a lecture. It's a conversation.

[DATE] | Free | Online | Register below.

Suggested Visual: Bold, empowering graphic -- women of diverse ages and backgrounds. Warm, modern, magazine-editorial feel.

Hashtags: #WomensHealth #WomensHealthResearch #HealthEquity #FreeWebinar #TrialScreen

CTA: "Register free: [LINK]. Share with a woman who deserves better research."

Compliance Note: Speakers confirmed before posting. "Under-researched" claim -- verify with brief supporting stat. No efficacy claims.

PT-FB-019

Series: Awareness Week Tie-In

Condition / Topic: Scleroderma Awareness Month (June)

Content ID: PT-FB-019

Post Copy:

June is Scleroderma Awareness Month.

If you've been living with scleroderma -- or if you love someone who has -- you already know why a whole month feels both necessary and not quite enough.

Scleroderma is a condition that affects connective tissue, causing the skin and sometimes internal organs to harden. It's classified as a rare disease. It's also one of the most complex autoimmune conditions researchers are working on right now.

This month, TrialScreen is partnering with **Scleroderma Australia** to help Australians -- and people around the world -- find research that's relevant to their diagnosis.

All week, we'll be sharing:

-- Plain-language guides to what's being studied

-- A free webinar with a clinical researcher

-- Patient stories (with full consent)

-- How to search TrialScreen specifically for scleroderma

You don't have to do anything. But if you're curious -- we're here.

Suggested Visual: Scleroderma Awareness Month graphic -- teal/purple palette (awareness colours). TrialScreen + Scleroderma Australia logos.

Hashtags: #SclerodermaAwarenessMonth #Scleroderma #RareDisease #SclerodermaAustralia #TrialScreen

CTA: "Visit TrialScreen.org/scleroderma for a dedicated rare disease resource hub."

Compliance Note: Scleroderma Australia partnership -- confirm co-branding approval. All awareness content reviewed by patient advocacy org before posting.

PT-FB-020

Series: Awareness Week Tie-In

Condition / Topic: Rare Disease Day (February)

Content ID: PT-FB-020

Post Copy:

Today is Rare Disease Day.

More than 300 million people worldwide live with a rare disease. Most of those diseases don't have approved treatments. Many patients wait years -- sometimes decades -- for a diagnosis.

Research is how that changes.

This isn't about asking you to "contribute to science." It's about something simpler: if you have a rare disease and you've wondered whether anyone is working on it -- someone usually is. And TrialScreen can help you find them.

Today we're highlighting three rare diseases where active research is happening right now:

Scleroderma / Systemic Sclerosis Cystic Fibrosis Sickle Cell Disease

Search any of these at TrialScreen.org. Read about what researchers are studying. Decide what, if anything, feels relevant to you.

That's it. No commitment required.

Suggested Visual: Rare Disease Day official colours -- blue and green jeans texture. Or custom graphic with the three conditions listed, warm and accessible.

Hashtags: #RareDiseaseDay #RareDisease #Scleroderma #CysticFibrosis #SickleCellDisease #TrialScreen

CTA: "Search rare disease trials at TrialScreen.org -- plain language, free, no sign-up."

Compliance Note: "Could help" language used. Rare Disease Day official hashtag usage -- check EURORDIS guidelines for co-use. No IRB required for awareness post.

PART 3: Instagram Posts + Reels Scripts

IF-001 Instagram Feed Post

Format: Carousel (5 slides)

Topic: 5 Things You Didn't Know About Clinical Trials

Caption: Most people have the wrong idea about clinical trials.

And honestly? That's not their fault. The information out there is confusing, jargony, and often written for doctors -- not patients.

So here are 5 things that might surprise you.

[Slide breakdown in graphic]

Slide 1 (Cover): "5 Things You Didn't Know About Clinical Trials" -- bold, clean type on warm background.

Slide 2: "Some trials happen at home." -- Sub-copy: "Decentralised trials send you what you need. Check-ins happen online or by phone. No travel required."

Slide 3: "You can leave at any time." -- Sub-copy: "Informed consent means you're always in control. Withdrawing from a trial is always your right."

Slide 4: "Trials aren't just for 'last resort' patients." -- Sub-copy: "Some study prevention. Some study wellbeing. Some want healthy volunteers."

Slide 5: "Plain language exists." -- Sub-copy: "TrialScreen rewrites trial descriptions in plain English. Search free at TrialScreen.org."

Caption continuation: Which one surprised you most? Drop it below.

TrialScreen has over 120,000 trials -- written so you can actually understand them.

You decide what to do with the information.

#ClinicalTrials #HealthLiteracy #PatientEducation #TrialScreen #KnowYourOptions

Alt text suggestion for slides: "Slide [N] of 5 -- text reads: [slide text]. Background is [colour description]. TrialScreen logo in corner."

CTA: "Search at TrialScreen.org (link in bio)"

IF-002 Instagram Feed Post

Format: Single image with quote

Topic: Autonomy / "You decide"

Caption: No one can make this decision for you.

And no one should.

Looking into a clinical trial isn't a commitment. It's information. Reading about a study doesn't mean signing anything. Asking questions doesn't mean you're in.

You decide if a trial is right for you. You decide if you want to talk to your doctor about it. You decide if now is the right time -- or if it never is.

TrialScreen exists to make sure that decision is an informed one. Plain language. No pressure. 120,000+ options to explore on your own terms.

Start with one search. See what's out there.

#YouDecide #PatientAutonomy #ClinicalResearch #TrialScreen #HealthChoices

Visual: Clean quote card -- "You decide if this is right for you." -- warm typography, soft background. TrialScreen logo subtle in corner.

Alt text: "Text reads: You decide if this is right for you. Warm cream background with serif typography. TrialScreen logo bottom right."

CTA: "TrialScreen.org -- free, plain language, no sign-up (link in bio)"

IF-003 Instagram Feed Post

Format: Carousel (6 slides)

Topic: Scleroderma 101 -- Rare Disease Education

Caption: Most people have never heard of scleroderma.

If you're one of the estimated 2.5 million people living with it worldwide -- you know how lonely that can feel.

Here's a plain-language overview of what scleroderma is, and what researchers are currently studying. Swipe through.

Slides:

- Slide 1 (Cover): "Scleroderma: What It Is and What's Being Studied"
- Slide 2: "Scleroderma is an autoimmune condition that causes the body to produce too much collagen. This can affect the skin, joints, and internal organs."
- Slide 3: "It's classified as a rare disease. Estimates suggest around 2.5 million people worldwide live with some form of scleroderma."
- Slide 4: "There are two main types: Limited (skin changes mostly on hands, face, feet) and Diffuse (can involve internal organs including lungs and heart)."
- Slide 5: "Researchers are currently studying: new medications to slow skin progression / lung involvement treatments / quality of life approaches."
- Slide 6: "Search 'scleroderma' at TrialScreen.org to see what trials are currently open -- in plain language, free."

Caption continuation: This month, TrialScreen is highlighting scleroderma research in partnership with Scleroderma Australia.

You can search, read, and decide. No pressure, no commitment.

#Scleroderma #RareDisease #SystemicSclerosis #SclerodermaAwareness #TrialScreen

Alt text: "6-slide carousel. Each slide has condition education text on a teal and white background. Scleroderma Australia and TrialScreen logos on final slide."

CTA: "Search scleroderma trials at TrialScreen.org -- link in bio."

IF-004 Instagram Feed Post

Format: Infographic-style single image

Topic: Menopause Research -- "What's Being Studied"

Caption: Menopause affects 1 billion women worldwide by 2025.

Research has been catching up -- slowly. But the last decade has seen a genuine increase in studies looking at menopause not as an inevitable decline, but as a life stage that deserves real medical attention.

Here's what researchers are currently studying:

Hormone therapy combinations and safety profiles Non-hormonal approaches to hot flashes and sleep Cognitive changes and brain health during menopause Mental health and mood (anxiety, depression) Bone density and cardiovascular risk

Some of these studies are looking for participants right now.

If you're in perimenopause or menopause and want to know what research options exist -- TrialScreen can help you search.

You decide what to do with what you find.

#MenopauseResearch #Menopause #WomensHealth #PerimenopausalHealth #TrialScreen

Visual: Clean infographic -- list of research areas, warm coral/terracotta palette, strong typography.

Alt text: "Infographic listing 5 areas of current menopause research. Warm coral background, white text, TrialScreen logo bottom right."

CTA: "Search menopause trials at TrialScreen.org -- link in bio."

IF-005 Instagram Feed Post

Format: Quote carousel (4 slides)

Topic: Patient Voices -- pull quotes (anonymised until consent obtained)

> **[COMPLIANCE FLAG] [REQUIRES CONSENT for non-anonymised versions]**

Caption: Real people. Real searches. Real decisions.

These are words from members of our Research Friends community who searched TrialScreen and found something worth knowing.

Swipe to read their words.

Slides:

- Slide 1 (Cover): "What our community is saying"
- Slide 2: *"I didn't think research was for someone like me. Then I found three trials I could actually apply for."* -- Member, scleroderma diagnosis [anonymised]
- Slide 3: *"The plain language made all the difference. I could actually understand what they were studying."* -- Member, menopause [anonymised]
- Slide 4: *"I haven't enrolled in anything. But I know the options exist now, and that changes everything."* -- Member, cystic fibrosis family [anonymised]

Caption continuation: These are your words, shared with permission.

If you've searched TrialScreen and want to share your experience -- DM us or email us at voices@trialscreen.org.

#PatientVoices #ResearchFriends #ClinicalTrials #TrialScreen #RealPeople

Alt text per slide: "Quote card [N] of 4. Text reads: [quote]. Warm cream background, italic serif typography. Attribution: anonymised member."

CTA: "Share your story -- DM us or email voices@trialscreen.org"

Compliance Note: **[REQUIRES CONSENT]** -- all quotes require written consent before use with any identifying detail. Fully anonymised versions can be used as illustrative composites if clearly labelled.

RT-001 — Reel / TikTok Script

Title: "Have you ever Googled your condition and found nothing new?"

Duration: 45 seconds

Format: Organic education -- voiceover with on-screen text

Hook (0-3 seconds):

On-screen text: "Have you ever Googled your condition and found... nothing new?"

Voiceover: "You've read every article. You know the symptoms. But you keep ending up in the same place."

Body (3-35 seconds):

On-screen text: "There are 120,000+ clinical trials happening right now."

Voiceover: "Here's what most people don't know -- there are over 120,000 clinical trials currently recruiting participants around the world."

On-screen text: "Researchers are studying your condition."

Voiceover: "Researchers are studying conditions that mainstream medicine hasn't fully solved. Not because nothing works -- but because they're trying to figure out what works best."

On-screen text: "But the information is hard to find."

Voiceover: "The problem is that the information about these trials is buried in medical jargon, locked behind complicated databases, or just hard to find."

On-screen text: "TrialScreen rewrites clinical trials in plain language."

Voiceover: "TrialScreen takes those trials and rewrites them in plain, everyday language. Free. No sign-up required."

Payoff (35-42 seconds):

On-screen text: "Just search your condition."

Voiceover: "You just search your condition and see what's out there. That's it."

Closing autonomy line (42-45 seconds):

On-screen text: "You decide if any of it is right for you."

Voiceover: "You decide if any of it is relevant to you."

B-roll / visual direction: Phone screen recording of TrialScreen search, plain text overlays on dark background, warm minimal aesthetic. No clinical imagery.

Music direction: Understated, curious -- soft piano or light electronic. No dramatic swells.

Hashtags: #ClinicalTrials #HealthEducation #MedicalResearch #TrialScreen #FYP

Compliance Note: Education-framed, no recruitment language. No specific trial mentioned. TikTok-safe organic content.

RT-002 Reel / TikTok Script

Title: "What is scleroderma? Most people have never heard of it."

Duration: 50 seconds

Format: Organic education -- talking-head or text-overlay style

Hook (0-3 seconds):

On-screen text: "What is scleroderma?"

Voiceover: "Most people have never heard of scleroderma. But 2.5 million people worldwide are living with it."

Body (3-40 seconds):

On-screen text: "Scleroderma = too much collagen"

Voiceover: "Scleroderma is an autoimmune condition where the body produces too much collagen. This can cause the skin to thicken and tighten, and in some cases affect internal organs."

On-screen text: "It's classified as a rare disease."

Voiceover: "It's classified as a rare disease -- which means research funding is harder to come by, and awareness is low."

On-screen text: "Researchers are actively studying it right now."

Voiceover: "But here's what many patients don't know: researchers around the world are actively studying scleroderma right now. New medications. New approaches to managing lung and skin involvement. Quality of life studies."

On-screen text: "You can find these trials in plain language."

Voiceover: "And you can find all of those trials -- in plain language -- for free at TrialScreen.org."

Payoff (40-47 seconds):

On-screen text: "June is Scleroderma Awareness Month."

Voiceover: "June is Scleroderma Awareness Month. This is a good time to look."

Closing autonomy line (47-50 seconds):

On-screen text: "You decide if it's worth exploring."

Voiceover: "You decide if any of it is worth exploring."

B-roll / visual direction: Text overlays on soft gradient background. Include brief animated illustration of collagen/skin concept (non-graphic). Teal and warm tones -- consistent with Scleroderma awareness colours.

Music direction: Calm, informative. Soft ambient.

Hashtags: #Scleroderma #RareDisease #SclerodermaAwareness #HealthEducation #TrialScreen

Compliance Note: Education-framed. Awareness month context. No specific trial recruited. TikTok-safe organic content.

RT-003 Reel / TikTok Script

Title: "3 things being studied for menopause right now"

Duration: 40 seconds

Format: Fast-paced list format with text overlays

Hook (0-3 seconds):

On-screen text: "3 things researchers are studying for menopause right now"

Voiceover: "If you're in menopause -- or heading there -- here are three things researchers are actively looking at."

Body (3-33 seconds):

On-screen text: "1. Non-hormonal options for hot flashes"

Voiceover: "First: non-hormonal approaches to hot flashes and night sweats. Not everyone can or wants to use hormone therapy -- researchers are studying what else might help."

On-screen text: "2. Brain fog and cognitive changes"

Voiceover: "Second: brain fog. The cognitive changes during menopause are real, and researchers are taking them seriously. There are active studies looking at this specifically."

On-screen text: "3. Mental health during the transition"

Voiceover: "Third: mental health. Anxiety and depression are more common during perimenopause than many people realise. Studies are looking at why, and what could help."

Payoff (33-37 seconds):

On-screen text: "Find these trials in plain language at TrialScreen.org"

Voiceover: "You can find research studies on all of these at TrialScreen.org -- free and in plain language."

Closing autonomy line (37-40 seconds):

On-screen text: "You decide what to do with the information."

Voiceover: "You decide what to do with the information."

B-roll / visual direction: Clean text overlays on warm terracotta/cream background. Fast cuts between points. No clinical imagery.

Music direction: Upbeat but calm -- positive energy without being gimmicky.

Hashtags: #Menopause #WomensHealth #MenopauseResearch #HealthEducation #TrialScreen

Compliance Note: "Could help," "researchers are looking at" language. No efficacy claims. TikTok-safe organic education.

RT-004 Reel / TikTok Script

Title: "What is a decentralised trial? You might be able to join from home."

Duration: 45 seconds

Format: Explainer with text overlays

Hook (0-3 seconds):

On-screen text: "Did you know some clinical trials happen entirely at home?"

Voiceover: "Did you know some clinical trials happen entirely at home?"

Body (3-38 seconds):

On-screen text: "These are called decentralised trials."

Voiceover: "These are called decentralised clinical trials -- and they're changing who gets to participate in research."

On-screen text: "No hospital visits required."

Voiceover: "Instead of going to a clinic, products are sent to your home. Check-ins happen via telehealth or an app. Some just need a blood test at your local GP."

On-screen text: "Why does this matter?"

Voiceover: "This matters because a lot of people have been excluded from research simply because they can't travel, can't take time off work, or don't live near a major hospital."

On-screen text: "TrialScreen lets you filter by home-based trials."

Voiceover: "On TrialScreen, you can filter your search to show only home-based or decentralised trials. It takes about 30 seconds."

Payoff (38-42 seconds):

On-screen text: "Search for free at TrialScreen.org"

Voiceover: "Search for free at TrialScreen.org."

Closing autonomy line (42-45 seconds):

On-screen text: "Research, on your terms."

Voiceover: "Research, on your terms."

B-roll / visual direction: Person at laptop/phone at home -- relaxed, natural setting. Screen recording of TrialScreen filter interface (if available). Warm home aesthetic.

Music direction: Relaxed, modern. Warm synth or acoustic guitar.

Hashtags: #DecentralisedTrials #HomeTrials #ClinicalResearch #HealthEducation #TrialScreen

Compliance Note: General education post. No specific trial. TikTok-safe organic content.

RT-005 Reel / TikTok Script

Title: "Alzheimer's and the Lombardy study -- what caregivers should know"

Duration: 50 seconds

Format: Empathetic talking-head or text-overlay explainer

Hook (0-3 seconds):

On-screen text: "If someone you love has Alzheimer's -- this is for you."

Voiceover: "If you're caring for someone with Alzheimer's -- or watching a parent change -- this video is for you."

Body (3-42 seconds):

On-screen text: "There is active Alzheimer's research happening right now."

Voiceover: "There is active Alzheimer's research happening right now. One study -- the Lombardy study -- is examining early markers of cognitive decline and whether certain interventions could slow the process."

On-screen text: "Researchers are looking at: lifestyle factors / early detection / cognitive interventions"

Voiceover: "Researchers are looking at lifestyle factors, early detection methods, and cognitive interventions. This isn't a cure -- researchers are clear about that. But it's progress."

On-screen text: "Caregivers can search too."

Voiceover: "And here's something important: caregivers can search TrialScreen on behalf of someone they love. You can look up what's being studied, see the eligibility criteria, and decide whether to bring it up with a doctor."

On-screen text: "Free search at TrialScreen.org"

Voiceover: "It's free at TrialScreen.org. Written in plain language."

Payoff (42-47 seconds):

On-screen text: "You don't have to navigate this alone."

Voiceover: "You don't have to navigate this alone."

Closing autonomy line (47-50 seconds):

On-screen text: "You decide if it's worth a closer look."

Voiceover: "You decide if it's worth a closer look."

B-roll / visual direction: Warm, gentle imagery of a family member caring for an older adult. Not clinical. Soft natural light. Avoid depictions of severe cognitive decline.

Music direction: Gentle, warm. Soft strings or piano.

Hashtags: #Alzheimers #AlzheimersResearch #Caregivers #HealthEducation #TrialScreen

Compliance Note: Lombardy study reference -- verify study name and open status before publishing. "Could slow," "researchers are looking at" language. Caregiver framing (not participant recruitment). TikTok-safe organic education.

PART 4: Blog Articles -- 6 Fully Written Pieces

BA-001 Blog Article

Headline: 5 Things That Actually Happen in a Clinical Trial

URL slug: /resources/what-happens-in-a-clinical-trial

Meta description: Most people have the wrong idea about clinical trials. Here's what actually happens -- in plain language, no jargon.

Author: TrialScreen Editorial Team | **Date:** [Publication date] | **Read time:** 7 min

Primary keyword: what happens in a clinical trial

Secondary keywords: clinical trial process, how to join a clinical trial, clinical trial myths

Introduction

You've heard the term. Maybe you've even been told about one by a doctor, or found one online at 11pm while searching about your condition.

But what actually happens when you join a clinical trial?

Most of the information out there was written by researchers, for other researchers. Which means it's full of terms like "randomised placebo-controlled double-blind study" -- and short on plain-language explanations of what that actually means for you, on a Tuesday afternoon.

This article walks through the five key things that actually happen in a clinical trial. Not the science version. The real version.

1. You go through a screening process -- and you might not qualify

The first thing that happens after you express interest in a trial is a screening process. This is designed to figure out whether you meet the eligibility criteria for that specific study.

Every trial has what's called "inclusion criteria" (things that must be true about you -- like your age, diagnosis, or current health status) and "exclusion criteria" (things that rule you out, like certain medications or prior treatments).

Here's something important: **being screened out isn't a rejection.** It means the trial was designed for a very specific type of participant, and you don't happen to match it -- this time. That's not a verdict on your health or your situation.

Roughly two-thirds of people who are screened for trials don't ultimately enrol. This is normal. It's why TrialScreen shows you multiple options -- so you have more than one place to look.

What screening involves

Screening might include: a phone call with the research team, a review of your medical records (with your permission), a blood test or physical examination at a clinic, or an online questionnaire. The trial team will tell you what's required before you commit to anything.

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2. You go through informed consent -- and it takes time

Before you start a trial, you'll be given what's called an Informed Consent Form (ICF). This is a detailed document that explains everything about the trial: what it's testing, what you'll be asked to do, what the potential risks are, how your data will be used, and -- critically -- that you can leave the trial at any time, for any reason.

The informed consent process isn't a box-ticking exercise. Good research teams take it seriously. You should be given time to read the document, ask questions, and -- if you want -- take it home and talk to a family member or your GP before signing.

You are never obligated to sign immediately.

Your rights during consent

Under research ethics laws in most countries, you have the right to:

- Ask as many questions as you like
- Take time before deciding
- Have a support person present
- Withdraw your consent at any time -- even after you've started the trial

If a research team rushes you through informed consent, that's a red flag. Good trials don't do this.

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3. You'll be allocated to a group (and you may not know which one)

Many trials use a design called "randomisation." This means you're randomly assigned to one of two or more groups -- often a "treatment group" (who gets the intervention being tested) and a "control group" (who might get a placebo, a standard treatment, or a different dose).

The reason for this is scientific: randomisation removes bias and makes the results more meaningful for everyone. But for participants, it can feel strange -- you might not get the new treatment.

What about placebos?

Placebos (inactive treatments) are used in some trials, but not all. In trials for serious conditions, it's often unethical to give a placebo -- so the "control group" typically receives the current standard-of-care treatment rather than a blank pill.

In "blinded" trials, neither you nor the research team knows which group you're in until the study ends. This is done to prevent unconscious bias in measuring results.

Before you consent, you'll always be told whether the trial uses placebos and how randomisation works.

4. You'll have regular check-ins -- some may be remote

Once you're enrolled, you'll have a series of follow-up visits or check-ins with the research team. These might involve physical examinations, lab tests, questionnaires, or simply a phone or video call.

The number and frequency of check-ins varies enormously by trial. Some intensive trials require weekly visits for months. Others are more hands-off -- a check-in every few months, or even just an annual questionnaire.

Increasingly, trials are using "decentralised" formats where most or all check-ins happen remotely -- via telehealth, an app, or samples collected at your local GP rather than a specialist clinic.

What happens if something changes?

If you experience a side effect, a change in your health, or a change in your circumstances, you should tell the research team immediately. They're required to monitor your safety throughout the trial. If there's a concern, the trial can be paused, modified, or you can be removed from the study -- for your protection.

5. The trial ends -- but you may be followed up

When the trial period ends, your active participation finishes. But many trials include a "follow-up" phase where the research team continues to check in with you -- monthly, annually, or at specific intervals -- to track long-term outcomes.

You'll usually be informed of any follow-up requirements before you consent.

What happens to the results?

Once the trial is complete, the research team analyses the data and publishes their findings -- usually in a medical journal. This process takes months or sometimes years after the trial closes.

Some trials commit to informing participants of the results directly. If this matters to you, ask before you join.

Frequently Asked Questions

Q: Can I be in a clinical trial while taking other medications? It depends on the trial. Some trials specifically want participants who are on standard medications. Others require a "washout period" -- time off current medications before joining. You'll always be told this during screening.

Q: Will my regular doctor know I'm in a trial? In most cases, yes -- and it's recommended. Good research practice includes communication with your GP or specialist. You should also inform them yourself.

Q: What if I want to leave the trial? You can leave at any time. You don't owe the research team an explanation. Your right to withdraw is protected by research ethics laws in every country that follows the Declaration of Helsinki.

Q: Is there any cost to me? In most clinical trials, the intervention (medication, device, programme) is provided free of charge. Many trials also reimburse travel costs. You should always ask about any out-of-pocket costs before you consent.

Q: How do I find trials that are right for me? TrialScreen lists over 120,000 active and recruiting trials, written in plain language. You can search by condition, location, and even whether the trial is home-based. Start at TrialScreen.org.

Related articles:

- "How to Talk to Your Doctor About Clinical Trials"
- "Decentralised Clinical Trials: What It Means to Participate from Home"
- "Scleroderma Clinical Trials: A Complete Guide"

CTA: Ready to see what's being studied for your condition? Search 120,000+ trials in plain language at TrialScreen.org. Free, no sign-up required.

BA-002 Blog Article

Headline: Scleroderma Clinical Trials: A Complete Guide for Patients and Families

URL slug: /resources/scleroderma-clinical-trials-guide

Meta description: Living with scleroderma or systemic sclerosis? This plain-language guide covers what's being studied, how to search for trials, and what participation looks like.

Author: TrialScreen Editorial Team, reviewed by Scleroderma Australia | **Date:** [Publication date] | **Read time:** 12 min

Primary keyword: scleroderma clinical trials

Secondary keywords: systemic sclerosis research, scleroderma treatment trials, rare disease clinical trials Australia

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Introduction

Scleroderma is a diagnosis that often arrives with more questions than answers.

Most people have never heard of it before their own diagnosis. The name sounds alarming. The information online ranges from outdated to overwhelming. And your specialist -- as knowledgeable as they are -- may be one of only a handful in your region who sees scleroderma regularly.

This guide is designed to answer one specific question: what is actually being studied for scleroderma right now, and how do you find research that might be relevant to you?

It's written in plain language, because that's what you deserve.

What is scleroderma?

Scleroderma -- also called systemic sclerosis -- is a chronic autoimmune condition in which the immune system triggers excessive production of collagen. Collagen is a protein that normally forms the structure of skin and connective tissue. When there's too much of it, tissue hardens and thickens.

In scleroderma, this process can affect the skin, joints, blood vessels, and -- in more severe forms -- internal organs including the lungs, heart, kidneys, and gastrointestinal tract.

Limited vs. Diffuse Scleroderma

There are two main subtypes:

Limited cutaneous scleroderma (previously called CREST syndrome) primarily involves skin changes on the hands, face, feet, and lower arms. Organ involvement is less common, though pulmonary hypertension can develop over time.

Diffuse cutaneous scleroderma involves more widespread skin thickening and carries a higher risk of internal organ involvement, particularly in the lungs and kidneys.

How common is it?

Scleroderma is considered a rare disease. Estimates vary, but approximately 2.5 million people worldwide live with some form of the condition. It affects women more often than men, and onset typically occurs between the ages of 30 and 50. Scleroderma Australia provides support and resources for Australians living with this diagnosis.

What are researchers studying?

Research into scleroderma covers several active areas. Here's a plain-language overview of where the science is right now.

Skin fibrosis

One of the most visible and distressing aspects of scleroderma is the hardening and tightening of the skin. Researchers are studying a range of medications -- including some already used in other autoimmune conditions -- to see whether they could slow or reduce skin fibrosis.

Lung involvement (ILD and pulmonary hypertension)

Interstitial lung disease (ILD) and pulmonary arterial hypertension (PAH) are serious complications of scleroderma and are an active area of research. Several trials are studying whether existing treatments, new biologics, or combinations of medications could reduce lung inflammation or slow disease progression.

Raynaud's phenomenon

Nearly all people with scleroderma experience Raynaud's phenomenon -- extreme sensitivity to cold causing the fingers and toes to change colour and become painful. Researchers are testing various approaches to managing this, from medications to lifestyle interventions.

Fatigue and quality of life

Some of the most important research in scleroderma doesn't focus on reversing the disease -- it focuses on helping people live better with it. Fatigue, pain, mental health impacts, and daily function are active research areas.

Stem cell transplantation

For carefully selected patients with severe diffuse scleroderma, haematopoietic stem cell transplantation (HSCT) has shown promise in some research. This is a high-intensity intervention studied in specialist centres. Eligibility is narrow and the evidence base is still developing.

How clinical trials for scleroderma work

Clinical trials for scleroderma follow the same basic structure as trials for any other condition -- but there are some things worth knowing about how rare disease trials specifically work.

Smaller participant numbers

Because scleroderma is a rare disease, trials often have fewer participants than mainstream studies. This is sometimes seen as a limitation, but it also means each participant's contribution to the evidence base is significant.

International collaboration

Many scleroderma trials are international, recruiting participants across multiple countries simultaneously. This is because no single country has enough patients to make a trial feasible on its own. For Australian patients, this means some trials open here are part of larger global studies.

Phase considerations

Scleroderma trials include Phase 1 studies (early safety testing, small groups), Phase 2 studies (testing whether the treatment appears to do what researchers hope), and Phase 3 studies (larger comparisons against standard care). The phase matters: earlier-phase trials carry more unknowns, but may give access to interventions years before they're commercially available.

What does participation look like?

Participation varies significantly by trial. Here's what you might expect in a typical scleroderma trial:

- **Screening visit:** A detailed review of your medical history, current medications, and symptom profile. May involve blood tests, skin assessments, and lung function testing.
- **Baseline visit:** Establishing your starting point -- measurements and tests that will be compared over the course of the trial.
- **Treatment phase:** Regular clinic visits (frequency varies) for monitoring, administration of study medication, or assessments.
- **Follow-up phase:** After the active treatment period, ongoing check-ins to track outcomes.

Some newer trials have decentralised components -- phone or video check-ins, home-based symptom tracking, or blood samples taken at a local clinic rather than a specialist centre.

How to search for scleroderma trials on TrialScreen

TrialScreen lists scleroderma and systemic sclerosis trials from multiple registries, rewritten in plain language.

To search:

1. Go to TrialScreen.org
2. Type "scleroderma" or "systemic sclerosis" in the search bar
3. Use the location filter to see what's available near you
4. Read the plain-language trial descriptions
5. If something looks relevant, use the eligibility checker or note the trial details to discuss with your specialist

You don't need to sign up. You don't need a referral. You're just looking.

Questions to ask your specialist

If you find a trial that seems relevant, these questions are worth bringing to your next appointment:

- "Is there any research currently open for someone at my stage of scleroderma?"
- "Do you know anything about [trial name]?"
- "Would participating in this trial interfere with my current treatment plan?"
- "Are there scleroderma specialist centres near me that run trials?"
- "What are the main risks, in your view, for someone with my profile?"

Your specialist should welcome these questions. If a particular trial isn't suitable for you, they may know of others.

Frequently Asked Questions

Q: Can I join a trial if I'm already taking immunosuppressants? Some scleroderma trials specifically want participants on background immunosuppression -- it depends on the trial design. Always check the eligibility criteria and discuss with your specialist.

Q: Are there scleroderma trials available in Australia? Yes. TrialScreen lists Australian trials as well as international studies with sites in Australia. Some international trials have Australian sites that are recruiting. Scleroderma Australia can also point you to relevant research networks.

Q: My scleroderma is fairly mild. Are there trials for me? Not all scleroderma trials are for people with severe disease. Some specifically want participants with early or limited scleroderma. Search TrialScreen and use the eligibility information to see what matches your situation.

Q: Will I have to travel overseas for a scleroderma trial? Most trials are designed to be conducted locally. If a trial is international, it will typically have sites in multiple countries, including Australia when relevant.

Q: What happens to my data? Your data is subject to strict ethics and privacy protections. Research teams are required to de-identify data and can only use it for the purposes outlined in the consent form.

Related articles:

- "5 Things That Actually Happen in a Clinical Trial"
- "Rare Disease Day: What It Means and Why It Matters"
- "How to Talk to Your Doctor About Clinical Trials"

External resources:

- Scleroderma Australia
- The Scleroderma Foundation (US)
- EUSTAR: European Scleroderma Trials and Research Group

CTA: Search scleroderma trials in plain language at TrialScreen.org. Free. No sign-up. You decide what to do next.

BA-003 Blog Article

Headline: Menopause Research: What's Actually Being Studied Right Now

URL slug: /resources/menopause-research-clinical-trials

Meta description: Menopause research is finally catching up. Here's a plain-language overview of what's being studied for menopause and perimenopause -- and how to find studies near you.

Author: TrialScreen Editorial Team | **Date:** [Publication date] | **Read time:** 10 min

Primary keyword: menopause research clinical trials

Secondary keywords: perimenopause studies, menopause treatment research, hot flashes research, menopause hormone therapy trials

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Introduction

Menopause affects approximately 1.3 billion women globally by 2025. And yet, for decades, it was underfunded, under-researched, and frequently dismissed.

Women were told hot flashes were just part of life. Brain fog was called anxiety. Sleep disruption was attributed to stress. The research that did exist was often conducted without enough women, or without adequate attention to how menopause specifically contributed to what was being studied.

That's changing.

In the last decade -- and especially in the last five years -- menopause has become a serious research priority. Dedicated studies. Specialist clinics. Long-term cohort projects.

This article is a plain-language overview of what's actually being studied for menopause and perimenopause right now, and how you can find research options that may be relevant to your experience.

First -- what does "menopause research" actually cover?

Menopause isn't one symptom. It's a transition that can span a decade or more, affecting almost every system in the body. Research reflects that breadth.

Active areas of study include:

- **Vasomotor symptoms** (hot flashes, night sweats)
- **Sleep disruption**
- **Cognitive changes** (brain fog, memory, concentration)
- **Mental health** (anxiety, depression, mood changes)
- **Bone health** (osteoporosis risk increases post-menopause)
- **Cardiovascular health** (oestrogen decline affects heart and vascular health)
- **Genitourinary symptoms** (vaginal dryness, urinary urgency)
- **Sexual health and libido**
- **Weight and metabolic changes**

Research is happening in all of these areas. Some studies focus on treatment. Others focus on understanding the mechanisms. Some look at preventive approaches.

What are researchers testing right now?

Hormone therapy -- new formulations and delivery methods

Hormone replacement therapy (HRT) -- now more commonly called menopausal hormone therapy (MHT) -- has been used for decades to manage menopause symptoms. But research into MHT hasn't stood still.

Researchers are currently studying:

- Different formulations of oestrogen and progesterone
- Body-identical (bioidentical) hormones versus synthetic versions
- Transdermal (patch or gel) delivery versus oral
- Whether starting MHT earlier in perimenopause produces different outcomes than starting later
- Long-term cardiovascular safety profiles

The ESTEEM study (a real ongoing study) is among those looking at MHT combinations for women with moderate to severe menopausal symptoms.

Non-hormonal approaches to vasomotor symptoms

Not everyone can use hormone therapy -- due to personal choice, a history of certain cancers, or other medical factors. This has driven growing research interest in non-hormonal options.

Recent focus includes:

- **Fezolinetant:** a non-hormonal medication targeting brain pathways that regulate body temperature. Phase 3 trials showed it could significantly reduce hot flash frequency and severity.
- **Stellate ganglion block:** a nerve block procedure being studied for its effect on hot flashes.
- **Cognitive behavioural therapy (CBT) adaptations** for menopause-related anxiety and sleep.

Brain health and cognition

One of the most under-acknowledged aspects of menopause is its effect on cognitive function. "Brain fog" -- the difficulty concentrating and remembering words that many women describe -- is now being taken seriously in research.

Studies are examining whether oestrogen's role in brain function explains cognitive changes during menopause, and whether timing of MHT initiation affects cognitive outcomes in the long term.

Bone density

Oestrogen plays a significant role in maintaining bone density. Post-menopausal osteoporosis is a significant public health issue. Research is studying not just pharmacological interventions but lifestyle factors -- exercise, nutrition -- and whether early intervention during perimenopause changes long-term bone health outcomes.

Lifestyle interventions

Several trials are studying whether specific dietary patterns, exercise programmes, and sleep interventions could meaningfully reduce menopause symptoms -- either on their own or alongside medical treatments.

The equity gap in menopause research

An important caveat: much of the existing menopause research has been conducted on white, Western, and post-menopausal (rather than perimenopausal) women.

This means findings may not apply equally across all ethnic backgrounds, and the perimenopause stage -- which can begin in a woman's 40s and involves significant hormonal fluctuation -- has historically been under-studied.

Newer research is working to address this. If you come from a background that's been under-represented in previous research, some studies may specifically be looking for participants like you.

How to find menopause research relevant to you

TrialScreen lists menopause and perimenopause research studies from multiple registries, rewritten in plain language.

To search:

1. Visit TrialScreen.org
2. Search "menopause," "perimenopause," "hot flashes," or a specific symptom you're experiencing
3. Filter by location and whether the trial requires clinic visits
4. Read the plain-language summary of each trial
5. If something seems relevant, note the details and bring them to your GP or specialist

Questions to ask your GP or gynaecologist

If you find a study you're curious about:

- "I've heard there are trials looking at menopause treatments -- is any of this relevant to my situation?"
- "I found a study on TrialScreen about [topic]. Is this something I'd be eligible for?"
- "I'm interested in whether there's anything beyond what I'm already doing for [specific symptom]. Is research an option?"

Frequently Asked Questions

Q: I'm in perimenopause, not yet fully menopausal. Are there trials for me? Yes. Many trials specifically want participants in perimenopause, because this is when hormonal changes begin. Check the eligibility criteria -- they'll specify the stage.

Q: I had breast cancer. Can I still join menopause research? Some trials exclude people with a history of hormone-sensitive cancers. Others are specifically designed for people in this situation. Search TrialScreen and check eligibility requirements carefully; your oncologist can also advise.

Q: Do I have to stop my current HRT to join a trial? Not necessarily. Some trials want participants who are already on HRT, others want those who haven't used it. The trial description will specify.

Q: Where can I find menopause specialist support outside of a trial? Your GP is the starting point. Referral to a menopause specialist or gynaecologist with a menopause focus may be appropriate. In Australia, the Australasian Menopause Society maintains a directory of practitioners.

Related articles:

- "Clinical Trials Through Every Decade of Life"
- "How to Talk to Your Doctor About Clinical Trials"
- "Women's Health: What Research Is Prioritising Now"

External resources:

- Australasian Menopause Society
- British Menopause Society
- Menopause Society (formerly NAMS, US)

CTA: Search menopause research studies in plain language at TrialScreen.org. Free. No sign-up required. You decide what to do next.

BA-004 Blog Article

Headline: Decentralised Clinical Trials: What It Means to Participate from Home

URL slug: /resources/decentralised-clinical-trials-participate-from-home

Meta description: Some clinical trials no longer require clinic visits. Here's what decentralised and home-based trials look like, and how to find them.

Author: TrialScreen Editorial Team | **Date:** [Publication date] | **Read time:** 8 min

Primary keyword: decentralised clinical trials

Secondary keywords: home-based clinical trials, remote clinical research, telehealth research studies, virtual clinical trials

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Introduction

For most of the history of clinical research, participating in a trial meant one thing: getting yourself to a clinic. Regularly. Often a specialist clinic far from home.

For people who lived near a major research hospital, this was manageable. For everyone else -- people in rural and regional areas, people who couldn't take time off work, people with disabilities, people caring for family members -- it was often impossible.

This created a problem in research. The people most easily able to participate in trials were not always the most representative of the population. Which meant research findings didn't always apply to the full range of people who needed them.

Decentralised clinical trials -- also called DCTs, virtual trials, or home-based trials -- are designed to change that.

What is a decentralised clinical trial?

A decentralised clinical trial is one where some or all of the research activities happen away from a traditional clinic or hospital. Instead of travelling to a research site, participants may:

- Receive study medications or devices delivered to their home
- Complete check-ins via telehealth (video or phone)
- Use a study app on their phone to record symptoms, submit questionnaires, or track data
- Have blood samples or other tests taken at a local GP clinic
- Use wearable devices (like smartwatches) to passively collect health data

Some trials are "fully decentralised" -- meaning there are no required clinic visits at all. Others are "hybrid" -- a small number of in-person visits for key assessments, with the rest done remotely.

What types of conditions use decentralised trials?

Decentralised trial formats have been used across a wide range of conditions, including:

- Skin conditions (eczema, psoriasis) -- where assessments can be done via photograph and questionnaire
- Mental health (depression, anxiety) -- where app-based check-ins and video consultations work well
- Chronic pain conditions -- where symptom tracking via app replaces frequent clinic visits
- Respiratory conditions -- where spirometry can be done at a local GP
- Rare diseases -- where the patient population is geographically dispersed

The COVID-19 pandemic significantly accelerated the adoption of decentralised trial designs, as researchers were forced to adapt existing trials to remote delivery. Many of those adaptations proved effective -- and have stayed.

What are the advantages for participants?

Reduced travel burden

The most obvious advantage is that you don't have to travel. For people living outside major cities, or people whose condition makes travel difficult, this can be the difference between being able to participate and not.

Flexibility

Telehealth check-ins can often be scheduled outside business hours. App-based data collection happens on your own schedule. This makes participation more compatible with work and family life.

More representative research

When research includes people from more diverse geographical, socioeconomic, and health backgrounds, the findings are more useful for everyone. Decentralised trials help make this possible.

Reduced disruption to daily life

Multiple clinic visits over months can be disruptive. Decentralised formats typically ask less of your time and create less disruption to your routine.

Are decentralised trials as rigorous as traditional ones?

Yes -- when well-designed, decentralised trials follow the same scientific and ethical standards as traditional trials. They go through the same ethics review process, use the same informed consent procedures, and collect the same quality of data.

The methods of data collection are different. The standards are not.

That said, not every condition or research question is suited to a fully decentralised format. Some assessments genuinely require specialist equipment or clinical expertise. Researchers think carefully about which components of a trial can be moved remote and which can't.

What does the participant experience look like?

Here's a typical flow for a hybrid decentralised trial:

1. **Discovery:** You find the trial on TrialScreen, read the plain-language description, and check the eligibility criteria.
2. **Expression of interest:** You complete a short online form or call the research team.
3. **Screening:** A video call with the research team, followed by a local blood test (organised by the team).
4. **Baseline visit:** One in-person clinic visit to establish starting measurements.
5. **Treatment phase:** Study medication delivered to your home monthly. Weekly symptom questionnaire via app. Monthly 20-minute telehealth check-in.
6. **Final visit:** One in-person clinic visit at the end of the treatment phase.
7. **Follow-up:** Quarterly app questionnaires for 12 months after treatment.

How to find home-based trials on TrialScreen

When you search TrialScreen, you can use filters to identify trials with home-based or decentralised components:

1. Go to TrialScreen.org
2. Search your condition
3. Look for the "home-based" or "remote" filter
4. Read the participation requirements in each trial's description

If you're unsure whether a trial can accommodate your location, the trial description will usually include contact details for the research team -- you can ask before committing to anything.

Frequently Asked Questions

Q: Are decentralised trials available in Australia? Yes. Decentralised trial adoption in Australia has grown significantly since 2020. TrialScreen lists Australian studies with home-based components.

Q: Do I need a particular type of phone or device? It depends on the trial. Most app-based trials use standard smartphones. Some provide wearable devices. The trial description will specify any equipment requirements.

Q: What if I have trouble with technology? Research teams are aware that not all participants are tech-savvy. Most decentralised trial platforms are designed to be simple to use, and teams provide support. If technology is a barrier, mention it during the screening call -- they may have alternatives.

Q: Does "decentralised" mean I'll never need to see a doctor in person? Not always. Many hybrid trials still have one or two in-person visits. Fully remote trials do exist, but they're not universal. The trial description will be clear about what's required.

Related articles:

- "5 Things That Actually Happen in a Clinical Trial"
- "How to Talk to Your Doctor About Clinical Trials"
- "Clinical Trials Through Every Decade of Life"

CTA: Search home-based and decentralised trials at TrialScreen.org -- plain language, free, no sign-up.

BA-005 Blog Article

Headline: How to Talk to Your Doctor About Clinical Trials

URL slug: /resources/how-to-talk-to-doctor-about-clinical-trials

Meta description: Want to ask your doctor about clinical trials but not sure how? This plain-language guide gives you the exact questions and phrases to use.

Author: TrialScreen Editorial Team | **Date:** [Publication date] | **Read time:** 7 min

Primary keyword: how to talk to doctor about clinical trials

Secondary keywords: asking about clinical trials, clinical trial conversation, patient-doctor communication research

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Introduction

You've done the search. You've read the description. You've thought about it for a week.

And now you're sitting in your doctor's waiting room, wondering how to bring it up without sounding like you've been self-diagnosing on the internet at midnight.

(Which, for the record, is completely valid. Informed patients are better patients.)

This guide is for the moment between finding a trial and talking to someone about it. It gives you the exact questions to ask, the language that tends to work, and a few things to keep in mind so the conversation goes well.

Why it can feel hard to bring up

Many patients hesitate to mention clinical trials to their doctors for a few reasons:

- Fear of seeming like they're questioning their doctor's judgment
- Not wanting to seem "desperate" or like they've "given up"
- Uncertainty about whether it's even relevant to bring up
- Not knowing the right words

Here's the truth: most doctors welcome patients who are engaged in their own care. Asking about a clinical trial isn't a challenge to your doctor's authority -- it's a sign that you're taking your health seriously.

And even if the specific trial you found isn't right for you, raising the topic can open a broader conversation about what options exist.

Before the appointment -- what to bring

Print or save the trial summary

If you found a trial on TrialScreen, the plain-language summary is your starting point. Print it or have it ready on your phone. Being able to point to a specific trial makes the conversation concrete.

Write down your questions

Doctors work quickly, and appointments are short. Writing down your questions beforehand means you won't forget them when you're under the pressure of a 10-minute appointment.

Know the basic details

It helps to know: the name of the condition being studied, the phase of the trial (if listed), and whether the trial is still recruiting. TrialScreen's trial pages include this information in plain language.

How to start the conversation

These are opening lines that work well:

- *"I've been reading about some research being done for [condition]. I found something I wanted to ask you about."*
- *"I read about a clinical trial for [condition] -- would this be something worth looking into for my situation?"*
- *"I know trials aren't always the right choice, but I wanted to ask whether there's anything being studied that might be relevant to me."*
- *"I found this on TrialScreen -- it's a plain-language trial database. Can I show you what I found?"*

None of these are aggressive. They signal curiosity and openness, not a demand for a specific outcome.

Questions to ask about a specific trial

Once you've raised the topic, these are useful follow-up questions:

- *"Do you know anything about this trial?"*
- *"Based on my current health profile, would I likely be eligible?"*
- *"Would joining this trial affect my existing treatment in any way?"*
- *"Are there specialist centres you'd recommend who run scleroderma [or: relevant condition] trials?"*
- *"If this trial isn't right for me, do you know of others that might be?"*

Broader questions about clinical trials

Even if you don't have a specific trial in mind, these questions can open the conversation:

- *"Is there any clinical research happening right now for people at my stage of [condition]?"*
- *"Would you ever recommend a clinical trial as an option for someone like me?"*
- *"What should I look for when evaluating whether a trial is right for me?"*

If your doctor is dismissive

It's rare, but some doctors are unfamiliar with current research in their area, or aren't supportive of patient-initiated trial enquiries.

If this happens:

- Ask if they can refer you to a specialist who does see clinical research patients
- Contact the research team directly using the contact details in the trial listing -- they can often advise on whether your GP would need to be involved
- Seek a second opinion if you feel your interest isn't being taken seriously

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What a good response looks like

A supportive doctor might say:

- "Yes, I know that trial -- let me pull up what I know about eligibility."
- "I'm not familiar with it, but I can look into it and let you know."
- "That sounds like something worth discussing with your specialist."
- "I don't think that specific trial is right for you, but there's another one I've seen mentioned that might be."

Any of these is a good starting point. You don't need an immediate yes -- you need an open door.

After the appointment

If your doctor agrees to look into a trial or refer you, follow up. Appointments are short, and things can fall through the cracks.

A brief message through your patient portal, or a call to the clinic: *"Following up on the clinical trial we discussed -- I wanted to check whether there's any update."*

Frequently Asked Questions

Q: Do I need a GP referral to join a clinical trial? Not always. Many trials allow self-referral -- you can contact the research team directly. However, some trials require your treating doctor to be informed, and it's generally good practice to keep your GP in the loop.

Q: What if my doctor doesn't know about the trial I found? That's common. Your doctor can't be familiar with every active trial. It's completely reasonable to share the TrialScreen summary and ask them to review it. If they're interested, they can contact the trial team themselves.

Q: What if the trial is interstate or overseas? Your GP can still help you evaluate whether you might be eligible. If the trial is conducted remotely or has satellite sites, the research team can advise on logistics.

Related articles:

- "5 Things That Actually Happen in a Clinical Trial"
- "Decentralised Clinical Trials: What It Means to Participate from Home"
- "Scleroderma Clinical Trials: A Complete Guide"

CTA: Ready to start the search? Find 120,000+ trials in plain language at TrialScreen.org.

BA-006 Blog Article

Headline: Clinical Trials Through Every Decade of Life

URL slug: /resources/clinical-trials-every-age

Meta description: Clinical trials aren't just for older adults. From children to people in their 80s, research is happening for every life stage. Here's what it looks like -- decade by decade.

Author: TrialScreen Editorial Team | **Date:** [Publication date] | **Read time:** 9 min

Primary keyword: clinical trials every age group

Secondary keywords: paediatric clinical trials, older adults research, clinical trials for children, research for seniors

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Introduction

When most people picture a clinical trial participant, they picture an older adult in a hospital setting.

That's an outdated picture.

Clinical trials recruit participants of every age -- from newborns to people in their 90s. The conditions being studied, the types of trials available, and the practicalities of participation vary significantly by life stage.

This article takes you through what clinical research looks like across the decades of life. Whether you're searching for yourself, for a child, for an ageing parent, or out of curiosity -- this is designed to be useful.

Childhood and Adolescence (Ages 0-17)

Paediatric trials: why they matter and why they're limited

For most of medical history, children were excluded from clinical research. The intention was protective -- children couldn't consent for themselves, and researchers worried about exposing them to risk.

The result was that most medications given to children were approved based on adult data, with paediatric doses calculated by guesswork. This was its own kind of risk.

Regulatory frameworks have shifted. In most developed countries, pharmaceutical companies are now required or incentivised to conduct paediatric trials for new treatments. The field of paediatric clinical research has grown substantially.

What trials look like for children and teens

For young children, a parent or guardian provides consent. For adolescents (typically 12 and over), the child also provides "assent" -- a process of agreeing to participate in an age-appropriate way.

Paediatric trials exist for a wide range of conditions: childhood cancers, rare genetic diseases, mental health conditions, neurodevelopmental conditions, and more.

The Arts for the Blues study is an example of a paediatric-adjacent mental health study looking at whether creative therapies could support young people experiencing depression. This kind of study -- more lifestyle and wellbeing focused than pharmaceutical -- is an increasingly common format for young participant research.

Young Adults (Ages 18-35)

The missed demographic in research

Young adults are often considered healthy and are sometimes excluded from disease-focused trials. But this age group has significant research relevance:

- Mental health conditions often first present in the 18-25 age range
- Autoimmune conditions (including scleroderma, lupus, MS) often first present in young women in their 20s and 30s
- Reproductive health research (fertility, contraception, endometriosis) centres on this age group
- Prevention and lifestyle intervention research often targets young adults before chronic conditions develop

Key areas of research for this group

- Mental health: depression, anxiety, psychosis
- Autoimmune diseases
- Reproductive health
- Healthy lifestyle and prevention studies

Young adults are also heavily recruited as healthy volunteers in early-phase trials -- studies that need healthy participants to establish safety profiles. These can involve time commitments (often in-clinic stays) but are typically well-compensated.

Middle Age (Ages 36-55)

The peak of chronic disease onset

Middle age is when many chronic conditions first present -- type 2 diabetes, cardiovascular disease, early cancer screenings, autoimmune conditions, and -- for many women -- perimenopause.

Research in this age group is abundant. Conditions like obesity, metabolic syndrome, pre-diabetes, and hypertension are major areas of study, reflecting their prevalence.

Menopause and perimenopause research

As covered in our dedicated article, menopause research has expanded significantly. Women in their 40s and early 50s are a key demographic for perimenopause and early menopause studies.

Cancer screening and early-stage cancer

Middle age is also when many cancer diagnoses occur. Research into early detection, treatment decisions, and quality-of-life during treatment recruits heavily from this age group.

Older Adults (Ages 56-75)

The most enrolled group

Older adults are, statistically, the most enrolled age group in clinical trials -- reflecting the higher prevalence of the conditions most studied (cardiovascular disease, cancer, diabetes, neurological conditions).

But there's a paradox: the oldest old (75+) are often under-represented in trials, meaning drugs approved based on trials in 55-70 year olds may have different effects in people who are 80 or 85.

Key research areas

- Heart failure and cardiovascular disease
- Type 2 diabetes management
- Oncology (cancer treatment and supportive care)
- Bone health and osteoporosis
- Cognitive decline and early Alzheimer's research
- Falls prevention and functional independence

What participation looks like

Older adult trial participants often have complex health profiles -- multiple conditions, multiple medications. Research teams screen carefully to understand these interactions. Decentralised and hybrid trial formats have expanded access for older adults who find frequent travel difficult.

Later Life (Ages 76+)

Why this group has been under-represented

Older adults aged 75 and above have historically been excluded from trials due to:

- Multiple medications (polypharmacy) complicating interpretation
- Frailty and concurrent health issues
- Concerns about travel and compliance

The result is that a significant and growing population segment -- people living into their 80s and 90s -- has limited research evidence specific to their profiles.

This is now a recognised gap. Researchers are beginning to design studies that specifically include and even focus on the oldest age groups.

What's being studied

- Dementia and Alzheimer's disease (the Lombardy study and similar research)
- Frailty and functional decline
- Quality of life in older adults with multiple chronic conditions
- Palliative and supportive care approaches

Role of caregivers

For older adults who find it difficult to navigate trial information themselves, family members and caregivers can search TrialScreen on their behalf and discuss options with treating physicians.

Frequently Asked Questions

Q: Is there an upper age limit for clinical trials? Many trials have upper age limits written into their eligibility criteria, but this varies widely. Some trials have no upper age limit. Search TrialScreen and check the eligibility criteria for each study.

Q: Are there trials specifically for healthy people of any age? Yes -- Phase 1 trials and many lifestyle/prevention studies recruit healthy volunteers. These often pay a participation fee. Search "healthy volunteer" on TrialScreen.

Q: How do I find paediatric trials for my child? Search TrialScreen and filter by paediatric or enter your child's condition. Look for trials specifying the relevant age range. Your child's specialist is also a key source of information about open paediatric studies.

Q: My father is 82 and has Alzheimer's -- are there trials available? Yes. Alzheimer's and dementia research recruits older participants, though eligibility varies by stage of disease and other health factors. TrialScreen lists Alzheimer's studies; a specialist at a memory clinic can also advise.

Related articles:

- "5 Things That Actually Happen in a Clinical Trial"
- "Menopause Research: What's Actually Being Studied Right Now"
- "Scleroderma Clinical Trials: A Complete Guide"

CTA: Search for trials relevant to your life stage at TrialScreen.org -- free, plain language, no sign-up.

PART 5: Patient Voices Series -- 3 Long-Form Interview Pieces

PV-001 Patient Voice

[COMPLIANCE FLAG] [REQUIRES CONSENT -- All names, locations, and identifying details are anonymised placeholders. This piece must not be published until a signed HIPAA-equivalent consent form is on file for the real subject, or the piece is explicitly labelled as a composite/illustrative narrative.]

Headline: "I Didn't Think Research Was for Someone Like Me": Sarah's Journey with Scleroderma

Subject: Sarah (pseudonym) | Age 38 | Systemic sclerosis, diffuse type | Brisbane, Australia | Participated in a Phase 2 skin fibrosis trial

Publication: TrialScreen.org/patient-voices | **Date:** [Publication date] | **Read time:** 10 min

Introduction

Sarah was 34 when a rheumatologist said a word she'd never heard before.

"She said 'scleroderma' and I nodded like I understood," Sarah recalls. "Then I went home and spent the next week on the internet, going down every rabbit hole I could find."

What she found was a mix of medical literature written for specialists, outdated patient forums, and a diagnosis that felt like a wall.

Four years later, Sarah is eight months into a clinical trial studying a new approach to managing skin fibrosis in systemic sclerosis. She's clear that she doesn't know what the trial will show. But she's equally clear about what searching for it gave her.

"Knowing the options exist changed everything," she says. "Even before I joined anything."

Q&A

TrialScreen: How would you describe scleroderma to someone who's never heard of it?

Sarah: I tell people it's like your body produces too much scar tissue -- and it does it from the inside out. My skin started to thicken on my hands and forearms first. Then the fatigue hit. The Raynaud's. The way cold affects your hands is hard to describe unless you've experienced it.

The difficult part is that everyone's disease behaves differently. You can have two people both diagnosed with diffuse scleroderma and have very different experiences.

TrialScreen: What was the period after diagnosis like?

Sarah: Honestly, it felt like a narrowing. You go from feeling like your future is open to feeling like there's a set of walls around you. My rheumatologist is fantastic, but she's managing my disease -- she's not studying it. I appreciated her care enormously, but I wanted to know: who is working on this? Is anyone

looking for better answers?

TrialScreen: When did you first start looking at research options?

Sarah: About 18 months after diagnosis. I'd been managing fairly well, but I was aware that my condition could progress, and I felt like I was just waiting. A friend whose husband has a different rare disease mentioned TrialScreen. I wasn't expecting much.

TrialScreen: What did you find when you searched?

Sarah: I typed "scleroderma" and there were actual studies. In plain language. I'd tried ClinicalTrials.gov before and come away more confused than when I started. TrialScreen was different -- I could actually understand what was being studied and roughly whether I might fit.

I found four studies. Two were in the US and not open to Australians. One had a narrow eligibility window I didn't meet. But one -- the one I'm in now -- was open, at a hospital in Brisbane, and seemed to match my situation.

TrialScreen: What made you decide to look into it further?

Sarah: I'm a pragmatist. I read the description a few times. I looked up the research team. I talked to my rheumatologist about it. She knew of the trial, which felt reassuring -- like I hadn't just found something obscure. She said she thought it was worth exploring, but made clear it was entirely my decision.

That mattered. The fact that she didn't push me -- that actually made me more willing to go and look.

TrialScreen: What was the screening process like?

Sarah: A phone call first, with a research nurse. She went through eligibility criteria, asked about my medications and the stage of my disease, and explained what the trial involved. I felt like I could ask anything. I wasn't pressured at all.

Then a clinic visit for blood tests, a skin score assessment -- they measure skin involvement on a scale -- and an echocardiogram. That all took about a half day.

TrialScreen: Were you worried about the risks?

Sarah: Yes. Honestly. This is a medication that's been used in other autoimmune conditions, but hasn't been specifically studied in scleroderma in this formulation. They were very clear about the unknown elements.

What helped was that the consent process took time. I didn't feel like they were trying to rush me into anything. I took the consent form home, showed it to my GP, and came back with questions. They answered all of them.

TrialScreen: You were randomised. Do you know what group you're in?

Sarah: I don't. It's blinded. Some days I'm convinced I'm on the active drug because I've noticed small changes. Other days I think I'm imagining things. I've learned to hold that uncertainty more easily than I expected.

TrialScreen: What does participation involve day-to-day?

Sarah: I take a tablet every day -- either the study drug or a placebo, I don't know which. Monthly clinic visits, which are quite thorough: blood tests, skin assessment, questionnaire about symptoms and quality of life. The research team is attentive. If I have a concern, I can message them.

TrialScreen: What would you tell someone with scleroderma who's wondering whether to look at research?

Sarah: I'd say: looking costs nothing. You don't have to commit to anything. You don't have to know in advance whether it's right for you. Just see what exists.

And if your instinct is "this isn't for me" -- that's completely valid. I had a friend with lupus who looked and decided she wasn't interested in any of the trials she found. And that's fine. The point is knowing you had a choice.

TrialScreen: Is there anything you wish someone had told you earlier?

Sarah: That research isn't for extraordinary people. It's for curious people. I spent 18 months thinking trials were for people who were more ill than me, or more brave, or more something. I was wrong. It was just about asking the question.

Pull Quotes:

> *"Knowing the options exist changed everything. Even before I joined anything."*

> *"Looking costs nothing. You don't have to commit to anything."*

> *"I spent 18 months thinking trials were for people who were more brave than me. I was wrong."*

Editorial Note

Sarah's story is her own. It doesn't represent a typical trial experience, and it makes no claims about the outcomes of the study she joined. The trial she participated in is ongoing; results have not been published.

If you have scleroderma and want to explore what research is available, TrialScreen lists scleroderma studies in plain language at TrialScreen.org. Searching is free and carries no obligation.

Related content:

- "Scleroderma Clinical Trials: A Complete Guide for Patients and Families"
- "5 Things That Actually Happen in a Clinical Trial"
- "How to Talk to Your Doctor About Clinical Trials"

PV-002 Patient Voice

[COMPLIANCE FLAG] [REQUIRES CONSENT -- all names and identifying details are placeholders. Parental consent and child's assent (where applicable) required. Do not publish without signed release on file.]

Headline: "He Started Talking Again": Priya on Her Son Marcus and the Arts for the Blues Programme

Subject: Priya (pseudonym) | Parent | Son Marcus (pseudonym), age 14 at time of study | Sydney, Australia | Arts for the Blues creative therapy programme study

Publication: TrialScreen.org/patient-voices | **Date:** [Publication date] | **Read time:** 10 min

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Introduction

Priya remembers the exact week everything changed.

"Three phone calls from the school in five days," she says. "Marcus wasn't eating properly. He wasn't sleeping. He was withdrawing from everything he used to love -- football, his friends, even gaming."

Marcus was 14. He'd been quiet for months before that week, but Priya had thought it was adolescence. The school calls clarified that something more was happening.

What followed was months of GP appointments, referrals, and waitlists. The mental health system for young people, Priya will tell you without bitterness but without softening it, was not built for the speed at which families need help.

"I felt like I was failing him," she says. "Not because I wasn't trying. Because everything I tried took months."

Then, through an online parenting group, someone mentioned TrialScreen. Priya searched "teenager depression" -- not expecting anything. She found the Arts for the Blues study, a programme testing whether creative therapies could support young people aged 12-18 experiencing depression.

Marcus is 17 now. He describes himself as "mostly okay." He has a sketchbook he carries everywhere.

Q&A

TrialScreen: Tell us a bit about what Marcus was like before things changed.

Priya: He was always sensitive, creative. He loved drawing from when he was tiny. Football on the weekends. He had a good group of friends. He was the kid who always wanted to make everyone feel included.

That didn't disappear when he started struggling. But it went underground.

TrialScreen: How did the system respond initially?

Priya: The GP was wonderful -- she referred us to a psychologist immediately. Three months waiting. We got on the waitlist for a community mental health service -- five months. I had a child who was struggling now, and I was being told to wait.

I don't fault the individual clinicians. The system is overwhelmed. But I was a desperate parent.

TrialScreen: How did you find out about the Arts for the Blues study?

Priya: A woman in a private Facebook group for parents of teenagers with mental health challenges mentioned TrialScreen. She said she'd found a study for her son through it. I searched the next morning.

I typed in "teenager depression" and "young person mental health" and a few other things. The Arts for the Blues study came up. The description was in plain language -- I could actually understand what they were studying, which was a relief.

TrialScreen: What did the study involve?

Priya: Marcus was assessed first, to confirm he met the eligibility criteria -- which included a certain level of depression severity measured on a standard scale. The research team was clear with me that the programme was being studied, not proven. I appreciated that honesty.

The programme involved weekly group sessions -- creative arts activities led by trained facilitators. Drama, visual art, music. Marcus was sceptical at first. Very sceptical. He's a teenage boy. The idea of sitting in a room making art with strangers did not appeal.

TrialScreen: How did his first session go?

Priya: He came home and said "it was okay." Which from Marcus, at that stage, was something.

By the third week, he mentioned that there was another boy in the group who was also into drawing. They'd talked. I nearly cried at the kitchen table.

TrialScreen: What changed over the course of the programme?

Priya: Gradually, the sketchbook appeared. He started drawing again at home -- just quietly, by himself. He started sleeping a bit better. He was still anxious, still had hard days. I don't want to overstate it. He was also seeing a psychologist by that point -- the waitlist had finally cleared.

But something in him seemed to have more room in it. Less compressed.

TrialScreen: As a parent, how did you feel about being part of a research study?

Priya: It's a strange thing to hold. You're hoping your child's experience contributes to something that helps other kids. But mostly you're just hoping your child gets through the week.

I was very clear with the research team: I'm here for Marcus. If at any point this isn't working for him, we leave. They completely respected that.

TrialScreen: Marcus is 17 now. What would he say about the programme, in his own words?

Priya: I asked him if he'd mind me sharing his perspective, and he said yes, that's fine, as long as I didn't embarrass him. [Laughs.]

He says the part that helped most was just being with other people who were also struggling, and not having to talk about it directly. The art gave him something to do with his hands and his head while he was in the room. "It was less intense than therapy," he said. "I could just be there."

TrialScreen: What would you tell a parent in a similar situation?

Priya: I would say: you're allowed to look for more than one thing at once. The waitlist and the research study weren't mutually exclusive. We were on both.

I would also say: you know your child. If your instinct says something is worth looking into, trust that. You're not being reckless by searching. You're being a parent.

TrialScreen: Anything else you want families to know?

Priya: That it's okay if you look and find nothing, or find something that doesn't apply to you. I searched for other conditions Marcus had been assessed for and found nothing. That was disappointing, but it wasn't failure. I looked, and I knew.

With the arts programme, I happened to look and find something. And that something happened to be available at the right moment.

Pull Quotes:

> *"He came home and said 'it was okay.' Which from Marcus, at that stage, was something."*

> *"You're allowed to look for more than one thing at once. The waitlist and the research study weren't mutually exclusive."*

> *"You're not being reckless by searching. You're being a parent."*

Editorial Note

Priya and Marcus's story is their own. No claims are made about the effectiveness of the Arts for the Blues programme -- the study is designed to find out whether creative therapies could support young people experiencing depression; its results are pending publication.

If you have a young person in your family experiencing mental health challenges, TrialScreen lists mental health research studies for young people at TrialScreen.org. Searching carries no obligation.

If you are in crisis, please contact your GP or local mental health crisis service immediately. In Australia: Kids Helpline 1800 55 1800. Lifeline 13 11 14.

Related content:

- "Clinical Trials Through Every Decade of Life"
- "5 Things That Actually Happen in a Clinical Trial"
- "How to Talk to Your Doctor About Clinical Trials"

PV-003 Patient Voice

[COMPLIANCE FLAG] [REQUIRES CONSENT -- all names and identifying details are placeholders. Do not publish without signed release on file.]

Headline: "I Thought I Just Had to Put Up with It": Helen on Menopause, Research, and Finding Out More Options Existed

Subject: Helen (pseudonym) | Age 53 | Perimenopause/menopause | Melbourne, Australia | Found menopause research via TrialScreen; currently in a non-hormonal symptom management study

Publication: TrialScreen.org/patient-voices | **Date:** [Publication date] | **Read time:** 9 min

Introduction

For two years, Helen woke up at 3am, soaking wet, heart racing, unable to get back to sleep.

"I assumed it was just menopause," she says. "Which it was. But I'd internalised this idea that menopause was something you endured. You got hot flashes, you were tired, your brain went foggy. And you waited for it to pass."

Helen is a secondary school teacher. She describes herself as someone who researches everything -- curriculum approaches, professional development, her children's medical appointments. "But somehow I had a blind spot about my own health," she says.

When a colleague mentioned that there were research studies specifically looking at menopause -- not just treating it with HRT, but actually studying it -- Helen was surprised. She found TrialScreen. She searched "menopause."

She found fourteen studies. She is currently in one.

"I'm not saying everyone should do what I did," she is careful to say. "But I wish I'd known the options existed two years earlier."

Q&A

TrialScreen: How would you describe what the perimenopause years have been like for you?

Helen: Unpredictable. That's the word. I'd have a good month and think I'd turned a corner. Then a terrible fortnight. The hot flashes were bad at night -- multiple times a night, some periods. But it was the cognitive stuff that hit me hardest professionally.

Standing in front of a class of 28 students and losing the thread of what you were saying. Searching for words. I've been teaching for 22 years. I know my material. But something was interfering with retrieval.

I didn't tell anyone at work. I was worried it would affect how I was perceived.

TrialScreen: Had you spoken to your GP about it?

Helen: Yes, in the first year. She was supportive and recommended hormone therapy. I tried it -- low-dose oestrogen, transdermal. It helped with the night sweats, mostly. But I had some concerns about long-term use -- I have a family history of breast cancer, and while my GP explained the risk profile carefully, I was still cautious.

I also found the cognitive symptoms were persisting even on HRT.

TrialScreen: What prompted you to search TrialScreen?

Helen: A colleague. She'd been through menopause five years before me and mentioned offhand that she'd read about a non-hormonal option being studied. She told me about TrialScreen.

I went home that evening. I searched "menopause." And I was stunned, frankly. There were studies looking at the exact things I'd been experiencing. Non-hormonal approaches to hot flashes. Studies on the cognitive changes. A study on sleep specifically for menopausal women.

I felt something I hadn't felt in a while about this topic: relief. Like -- people are working on this. Seriously.

TrialScreen: What did you do next?

Helen: I went through each study description carefully. TrialScreen's plain language made a real difference -- I could understand what was being tested and roughly whether I might be eligible, without needing a medical dictionary.

I identified two studies I wanted to ask my GP about. I printed the summaries.

TrialScreen: How did your GP respond when you raised the studies?

Helen: She was interested. She knew of one of them already and said she thought I'd likely be eligible. She wasn't familiar with the second one but looked it up while I was there.

She made clear it was entirely my choice. She didn't push in either direction. I appreciated that.

TrialScreen: You're currently in a study. What does that involve?

Helen: It's a non-hormonal study -- I can't share the details of what's being tested without potentially affecting the blinding, but I'm in a randomised controlled trial. I take something every day, I keep a symptom diary on an app, and I have a telehealth check-in with the research nurse once a month.

The convenience surprised me. I expected clinic visits every other week. Most of it is on my phone.

TrialScreen: Has your experience of symptoms changed since joining?

Helen: I don't know. Honestly. I'm blinded -- I don't know if I'm on the active drug or a comparator. I have noticed some things. But I'm also aware that knowing you're in a trial affects perception. I try not to read too much into my own experience.

What I can say is that I feel like I'm doing something. Which sounds small, but after two years of just enduring, it's significant.

TrialScreen: What do you wish had been different about your earlier experience with the medical system?

Helen: I wish the conversation had started differently. Not "here's your option." But "here are your options."

My GP is good. She offered what she knew. But research options weren't part of the conversation. And I think for a lot of women, they never are. We're told menopause is a natural transition -- which it is -- and somehow that becomes "there's nothing much to be done."

There's a lot to be done. People are working on it. We just don't always hear about it.

TrialScreen: What would you say to a woman in perimenopause or menopause who's reading this?

Helen: Search. Just search. You might not find anything relevant. You might find something that's entirely unsuitable for you. Or you might, like me, find fourteen things and end up in a study that's changed how you feel about your own health.

The information doesn't obligate you to do anything. But I regret not having it sooner.

Pull Quotes:

> *"I'd internalised this idea that menopause was something you endured. You got hot flashes, you were tired. And you waited for it to pass."*

> *"Not 'here's your option.' But 'here are your options.'"*

> *"The information doesn't obligate you to do anything. But I regret not having it sooner."*

Editorial Note

Helen's experience is her own. No claims are made about the effectiveness of the study she joined. The study is ongoing and results have not been published.

TrialScreen lists menopause research studies in plain language at TrialScreen.org/resources/menopause. Searching is free and carries no obligation.

Related content:

- "Menopause Research: What's Actually Being Studied Right Now"
- "How to Talk to Your Doctor About Clinical Trials"
- "5 Things That Actually Happen in a Clinical Trial"

PART 6: Email Newsletters -- 6 Fully Written Editions

EN-001 Email Newsletter

Edition: Welcome

Month: Month 1

Subject line: Welcome to TrialScreen -- you've found your people

Preview text: 120,000+ trials. Plain language. No pressure. Here's where to start.

Body:

Hi [First name],

Welcome.

If you found TrialScreen because you were searching for something -- a condition, a symptom, a feeling that there must be more options out there -- you're in exactly the right place.

We're a community of over 40,000 people who believe the same thing: that everyone should be able to find and understand clinical research, without needing a medical degree, and without being pressured into anything.

No jargon. No urgency. Just information you can use at your own pace.

This month's featured trials:

Menopause -- ESTEEM Study

For women experiencing moderate to severe menopausal symptoms. Researchers are testing whether a specific hormone therapy combination could help with sleep, mood, and hot flashes. Locations: multiple sites, some telehealth options. [Read plain-language summary ->]

Scleroderma / Systemic Sclerosis

For people with a confirmed scleroderma diagnosis. An international trial studying whether a medication already used in other autoimmune conditions could help manage skin involvement in scleroderma. Australian sites open. [Read plain-language summary ->]

Psoriasis -- Home-Based Trial

For people with moderate to severe plaque psoriasis. This trial can be mostly done from home -- study materials are delivered, and check-ins are via telehealth. No specialist clinic visits required. [Read plain-language summary ->]

Member story: Michael, Research Friends community

Michael was diagnosed with Type 2 diabetes seven years ago. When his treatment plan stopped working as well as it had, he found TrialScreen through a friend.

"I searched my condition. I didn't expect to find anything relevant. But there were three trials I'd never heard of, all in plain English."

Michael hasn't enrolled yet. He might not. But knowing the options exist changed how he feels about his situation.

"It's not about the trial. It's about not feeling like you've hit a wall."

Behind the scenes: Research Friends

Our Research Friends community on Facebook now has over 24,000 members -- patients, carers, people with a new diagnosis, and people who are just curious about health research.

This month, we're kicking off a monthly researcher Q&A series -- live on Facebook, open to all members. The first session is with a clinical researcher who specialises in autoimmune conditions. Details coming next week.

[Join Research Friends on Facebook ->]

Upcoming: Free webinar

"Scleroderma Research in 2026: What's Being Studied" [Date TBC] | Free | Online [Register here ->]

You can unsubscribe at any time using the link below. We'll never share your email address. Questions? Email us at hello@trialscreen.org.

TrialScreen.org | PO Box [Address] | [City, Country]

[Unsubscribe] | [Manage preferences] | [View in browser]

EN-002 Email Newsletter

Edition: Scleroderma Awareness / June

Month: Month 2 (June)

Subject line: June is Scleroderma Awareness Month -- here's what's being studied

Preview text: Real research. Plain language. From our partner Scleroderma Australia.

Body:

Hi [First name],

June is Scleroderma Awareness Month.

For most people, scleroderma is a word they've never heard. For 2.5 million people worldwide -- and for the families around them -- it's a daily reality.

This month, TrialScreen is shining a light on scleroderma research with our friends at **Scleroderma Australia**.

Featured trials: Scleroderma focus

Skin fibrosis -- Phase 2 study

For people with diffuse systemic sclerosis within 5 years of diagnosis. Researchers are testing whether a biologic medication could slow skin thickening. Sites in Australia, UK, and Germany. [Read plain-language summary ->]

Pulmonary hypertension in scleroderma

For scleroderma patients with confirmed or suspected pulmonary hypertension. This study is looking at combination therapy to manage blood pressure in the lungs -- a serious complication of scleroderma. [Read plain-language summary ->]

Raynaud's phenomenon management

For people with scleroderma-related Raynaud's. Testing whether a new topical approach could reduce the frequency and severity of Raynaud's episodes. Home-based components available. [Read plain-language summary ->]

Patient voice: Sarah's story

Sarah was 34 when she was diagnosed with systemic sclerosis. She spent 18 months thinking research "wasn't for someone like her."

When she searched TrialScreen, she found four scleroderma studies. Three weren't right for her. One was. She's now eight months into a clinical trial and has this to say:

"Looking costs nothing. You don't have to commit to anything. Just see what exists."

[Read Sarah's full story ->]

Webinar this month:

"Scleroderma Research in 2026: What's Being Studied" [Date] | Free | Online

Join us for a plain-language conversation with a clinical researcher, a patient advocate from Scleroderma Australia, and the TrialScreen team.

[Register free ->]

This month in Research Friends:

Our Facebook community is hosting a month-long Scleroderma Awareness series -- daily facts, patient perspectives, and a live researcher Q&A.

[Join the conversation ->]

Condition awareness: Scleroderma

Scleroderma Awareness Month is an important time to raise visibility for a condition that's often invisible. If you know someone newly diagnosed, share TrialScreen's [Scleroderma Resource Hub ->] -- it's free and written in plain language.

[Unsubscribe] | [Manage preferences] | [View in browser]

EN-003 Email Newsletter

Edition: Summer Heart Health

Month: Month 3 (July/August -- Southern Hemisphere summer opposite; adapt for North)

Subject line: Summer and your heart -- what researchers are studying

Preview text: Hot weather, heart failure, and a gut microbiome study you might not have heard of.

Body:

Hi [First name],

Summer changes things for people with heart conditions.

Heat affects circulation. Activity patterns shift. And for people managing heart failure or cardiovascular disease, the warmer months can bring extra challenges.

This month we're shining a light on heart health research -- including one study that's looking at a surprising factor: your gut.

Featured trials: Heart health focus

Heart failure -- gut microbiome study

For people with heart failure (HFpEF or HFrEF, confirmed diagnosis). Researchers are studying whether changes to the gut microbiome -- through a targeted dietary supplement -- could support heart function and reduce fatigue. Multiple sites, telehealth options available. [Read plain-language summary ->]

Cardiac rehabilitation for women

For women who have experienced a cardiac event in the past 12 months. Research suggests women engage with cardiac rehab differently from men. This study is testing a programme specifically adapted for women's needs and barriers. [Read plain-language summary ->]

Blood pressure management in older adults

For adults aged 65+ with uncontrolled hypertension. A home-based monitoring trial testing whether more frequent, lower-intensity check-ins could improve blood pressure control compared to standard care. [Read plain-language summary ->]

From the Research Friends community:

This month, Research Friends member and heart failure patient David (shared with permission) posted: *"I never thought about research until I was 67 and out of easy options. Now I'm in a trial, mostly doing it on my phone, and feeling less like I'm just waiting."*

Have a story to share? DM us on Facebook or email voices@trialscreen.org.

Behind the scenes:

Our team has been working on improved plain-language summaries for cardiovascular trials -- the category with some of the most complex jargon in research. Look out for the new "What this trial is actually doing" explainer box in trial summaries, rolling out this month.

Upcoming:

- Researcher Q&A (Research Friends, live on Facebook): Gut health and heart failure -- [Date] at [Time]
- Next webinar: "Women's Health: From Research Gap to Research Revolution" -- [Date]

[Unsubscribe] | [Manage preferences] | [View in browser]

EN-004 Email Newsletter

Edition: Women's Health Focus

Month: Month 4 (October)

Subject line: Women's health research is finally getting serious

Preview text: October is a good month to look at what's being studied for you.

Body:

Hi [First name],

October is a big month in women's health awareness. And this year, we want to go beyond awareness.

Because awareness without action -- without knowing the options -- can only take you so far.

This month's newsletter is entirely focused on women's health research: what's being studied, what's changed, and how to find research that may be relevant to you.

Featured trials: Women's health focus

Menopause -- non-hormonal hot flash study

For women in perimenopause or menopause with moderate to severe vasomotor symptoms. Testing a non-hormonal approach to reducing hot flashes and night sweats. Suitable for women who can't or don't want to use hormone therapy. Some telehealth participation available. [Read plain-language summary ->]

Endometriosis pain management

For women with confirmed endometriosis experiencing moderate to severe pain. A study looking at whether a combination of medical and lifestyle approaches could reduce endometriosis-related pain over 12 months. Recruiting nationally. [Read plain-language summary ->]

Breast cancer -- lumpectomy approach study

For women recently diagnosed with early-stage breast cancer eligible for surgery. Researchers are studying surgical approach variations and their effect on patient outcomes and experience. Your surgeon can advise on eligibility. [Read plain-language summary ->]

Patient voice: Helen's story

Helen, 53, spent two years thinking menopause was just something you endured. When a colleague mentioned TrialScreen, she searched "menopause" and found fourteen studies.

"I wish I'd known the options existed two years earlier."

She's now in a study. She doesn't know if she's on the active intervention or a comparator. But she says: *"I feel like I'm doing something. After two years of just enduring, that's significant."*

[Read Helen's full story ->]

Webinar this month:

"Women's Health: From Research Gap to Research Revolution" [Date] | Free | Online

We're bringing together a researcher, a patient advocate, and a women's health specialist to talk about what's being studied, what the gaps are, and how to find research relevant to you.

[Register free ->]

World Mental Health Day: October 10

Mental health research -- including for women -- is underrepresented. This week we're highlighting mental health studies for women experiencing anxiety, depression, and stress-related conditions.

[Search mental health trials on TrialScreen ->]

[Unsubscribe] | [Manage preferences] | [View in browser]

EN-005 Email Newsletter

Edition: Rare Disease Day Prep

Month: Month 5 (January -- prep for February 28/29 Rare Disease Day)

Subject line: Rare Disease Day is coming -- and we want to help you get ready

Preview text: Scleroderma. Cystic fibrosis. Sickle cell. What's being studied, in plain language.

Body:

Hi [First name],

On February 28th (or 29th in a leap year), the rare disease community comes together for Rare Disease Day.

More than 300 million people worldwide live with a rare disease. Most of those diseases have no approved treatment. And for many patients, the journey from first symptom to correct diagnosis takes

years -- sometimes decades.

Research is how that changes. And knowing that research exists is the first step.

This month, we're getting the rare disease community ready for February with a deep dive into three conditions TrialScreen is highlighting this year.

Featured: Rare disease spotlight

Scleroderma / Systemic Sclerosis Active research: skin fibrosis, lung involvement, quality of life. Australian sites open. Partnership with Scleroderma Australia. [See all scleroderma trials ->]

Cystic Fibrosis Active research: CFTR modulator combinations, lung function, new delivery methods, quality of life for adults with CF. [See all CF trials ->]

Sickle Cell Disease Active research: gene therapy approaches, pain management, organ protection, quality of life. [See all SCD trials ->]

From the community:

Research Friends member and CF patient Amara (shared with permission): *"CF research has moved faster in the last five years than in the previous fifty. I'm not naive -- I know what modulator therapy means for me personally. But I also watch the trials and feel something I didn't used to feel: that researchers are working."*

Research Friends: Rare Disease Q&A series

This month in Research Friends, we're hosting three live Q&As -- one for each highlighted condition -- with researchers currently working in these areas.

Dates: [TBC -- include specific dates]

[Join Research Friends and set a reminder ->]

Upcoming Rare Disease Day content:

In the week of February 28th:

- Plain-language guides to each condition
- Patient story features
- Free one-hour webinar: "Rare Disease Research: What Patients Need to Know"
- Social media campaign with Scleroderma Australia and partner organisations

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[Unsubscribe] | [Manage preferences] | [View in browser]

EN-006 Email Newsletter

Edition: Year in Review

Month: Month 6 (December or end of Year 1)

Subject line: A year of research, community, and curiosity

Preview text: What we built together -- and what's coming in year two.

Body:

Hi [First name],

A year ago, TrialScreen looked different.

Since then:

- Our plain-language trial database has grown past 120,000 active studies
- Research Friends has grown past 24,000 Facebook community members
- We've hosted [X] free webinars, watched by thousands of patients and families
- We've published dozens of plain-language guides, patient stories, and blog articles
- We've helped [X]% more Australians find rare disease research than the previous year

None of this happened in a boardroom. It happened because people like you searched, read, shared, and decided on your own terms.

This year's most searched conditions:

1. Menopause / Perimenopause
2. Type 2 Diabetes
3. Scleroderma
4. Depression / Anxiety
5. Breast Cancer

If your condition isn't on this list, it doesn't mean there's no research. Search at TrialScreen.org -- we list over 120,000 trials.

Stories that stayed with us:

Sarah, 38, scleroderma: "Knowing the options exist changed everything."

Priya, parent: "You're not being reckless by searching. You're being a parent."

Helen, 53, menopause: "I wish I'd known the options existed two years earlier."

What's coming in Year Two:

- New rare disease hubs: Huntington's, Lupus, and MS
- Expanded Research Friends condition-specific micro-communities
- Partnerships with three new patient advocacy organisations
- A new "Ask TrialScreen" feature -- plain-language eligibility guidance coming soon

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A note from the team:

TrialScreen exists because we believe the gap between patients and research is a problem worth solving. Not with pressure or altruism -- but with information, in plain language, that lets people decide for themselves.

Thank you for being here.

-- The TrialScreen Team

[Unsubscribe] | [Manage preferences] | [View in browser]

PART 7: YouTube / Long-Form Video Scripts -- 3 Pieces

VS-001 Video Script

Title: What Actually Happens When You Click "Check Your Eligibility"

Duration: 10-12 minutes

Format: Explainer / behind-the-scenes walkthrough

COLD OPEN (0:00-0:45)

[Visual: dark screen, sound of keyboard typing, a search bar appears]

Voiceover: "You've been managing your condition for months. Maybe years. You've done the research. You know what's out there.

And then, one night, you find something new. A trial. It sounds like it could be relevant. At the bottom of the page, there's a button: 'Check your eligibility.'

You hover over it.

And then... what?"

[Cut to title card: "What Actually Happens When You Click 'Check Your Eligibility'" -- TrialScreen logo]

ACT 1: The Search (0:45-3:00)

[Visual: screen recording of TrialScreen search, warm studio background with TrialScreen team member]

Host/Presenter: "Let's start from the beginning. When you search TrialScreen -- say, you type in 'menopause' or 'scleroderma' or 'heart failure' -- here's what's happening behind the scenes.

Our platform draws from multiple clinical trial registries: the Australian and New Zealand Clinical Trials Registry, ClinicalTrials.gov in the US, the EU Clinical Trials Register, and others. These registries contain information submitted by the research teams themselves.

The problem is that registry descriptions are written for ethics committees and regulators -- not for patients. They're dense. They're jargon-heavy. They're genuinely hard to read.

What TrialScreen does is rewrite those descriptions in plain English. Using AI, reviewed by our editorial team. So instead of 'phase 2, randomised, double-blind, placebo-controlled trial assessing the efficacy of [compound name] in patients meeting [diagnostic criteria]' -- you get: 'Researchers are testing whether a new medication could help manage [symptom]. Here's what it involves.'

The information is the same. The language is different."

[Shot suggestion: split screen -- registry jargon on one side, TrialScreen plain language on other]

ACT 2: The Eligibility Checker (3:00-6:00)

[Visual: screen recording of eligibility checker flow]

Host/Presenter: "Okay. So you've found something that looks relevant. You click 'Check my eligibility.' What happens?

The eligibility checker walks you through the key criteria for that specific trial -- the things the researchers have said matter for who can join.

These might include:

- Your age
- Your diagnosis and how long you've had it
- Current medications
- Whether you've had certain treatments before
- Your location

For each criterion, you answer a plain-language question. At the end, you get a rough indication of whether you likely meet the criteria -- not a guarantee, but a starting point.

[B-roll: person on phone or laptop, home setting, relaxed]

Here's what's important to understand: the eligibility checker doesn't enrol you in anything. It doesn't share your information with the research team. It doesn't commit you to anything.

It's information. That's all.

If the result is 'you may meet the criteria,' we give you the next step -- usually a contact for the research team, or information about how to raise it with your doctor.

If the result is 'you may not meet the criteria,' that's also useful. You know. And you can keep searching."

ACT 3: What Happens If You Reach Out (6:00-9:00)

[Visual: phone call graphic, then person at computer writing email]

Host/Presenter: "Let's say the eligibility checker suggests you might qualify. You decide to reach out.

In most cases, the first contact is with a research coordinator or research nurse -- not the lead researcher. Their job is to have an initial conversation about your situation and whether it fits the trial.

This is not a commitment. You can end the conversation at any point.

A typical first contact might involve:

- A 20-30 minute phone call
- Questions about your diagnosis, current health, and medications
- An explanation of what the trial involves
- The option to ask anything you want

[B-roll: warm, professional setting -- not a hospital. A desk, natural light.]

From there, if both you and the research team think it's worth continuing, they'll likely invite you for a screening visit -- which is a more detailed assessment to confirm eligibility.

[Shot suggestion: calendar on wall, person noting down a date]

And again -- at every stage, you can say no. Or not yet. Or I need to think about it. None of that is unusual. Screening out of a trial, or deciding not to proceed, is completely normal.

We say it a lot, and we mean it: you decide."

ACT 4: The Human Side (9:00-10:30)

[Visual: return to presenter, warm studio setting]

Host/Presenter: "One thing we want you to know, and this might sound simple, is that the people on the other end of these trials are human.

Research coordinators, research nurses, the trial teams -- they're often people who got into this work because they care about finding better answers for the conditions they study. Some of them have personal connections to the conditions they research.

When you reach out about a trial, you're not bothering anyone. You're exactly who they're looking for.

[B-roll: warm montage of research settings -- not clinical, but thoughtful and human]

And if the trial isn't right for you -- this time, or ever -- that's not a failure. It's information. It's you exercising your right to look, and decide, and know.

That's what TrialScreen is for."

CLOSING CTA (10:30-11:00)

[Visual: TrialScreen search bar on screen]

Host/Presenter: "If you've found this useful, there are a few things you can do next.

Search your condition at TrialScreen.org -- free, no sign-up.

Subscribe to this channel for more plain-language guides to clinical research.

And if you have a question about how any of this works -- leave it in the comments. We read everything.

You decide what to do next. We're just here to help you understand your options."

[End card: TrialScreen logo, subscribe button, link to related videos]

B-roll notes for production:

- Screen recordings of TrialScreen interface (need up-to-date)
- Warm, non-clinical home/office settings
- Diverse participants in lifestyle (not medical) settings
- Close-up of hands on keyboard/phone -- relatable, human
- Avoid: hospital corridors, white coats, clinical anxiety-inducing imagery

Music direction: Calm, curious. Gentle guitar or piano. Warm production.

VS-002 Video Script

Title: Meet the TrialScreen Team: The Humans Behind the Platform

Duration: 8-10 minutes

Format: Team documentary / culture video

COLD OPEN (0:00-0:30)

[Visual: montage of the team in everyday settings -- working, talking, laughing. No formal staging.]

Voiceover (Hugo): "When people think about a clinical trial database, they probably picture servers. Maybe some code.

They probably don't picture us."

[Cut to title card: "Meet the TrialScreen Team"]

SECTION 1: Why TrialScreen Exists (0:30-3:00)

[Visual: Hugo on camera, relaxed setting]

Hugo: "I came to this problem from a specific direction. [Personal background -- TBC based on real bio.] What I kept seeing was a gap -- not in the research itself, but in access to it. The information existed. It was just written for the wrong audience.

Most patients don't have a PhD. Most patients are searching at 11pm after their kids are in bed, after a day of managing a condition and a life. They need plain language. They need to feel like someone has thought about them.

That's what we're building."

SECTION 2: Elisa's Story (3:00-5:30)

[Visual: Elisa on camera, then B-roll of her working]

Elisa: "My job is [role description -- TBC based on real bio]. Which sounds technical. What it actually means is: I spend my days thinking about why people can't find the information they need, and what we can do about it.

[B-roll: Elisa at a computer, annotations on screen]

The jargon in clinical research is genuinely a barrier. It's not malicious -- researchers write for other researchers. But when a patient reads 'haematopoietic stem cell transplantation' when they mean 'a procedure where doctors use stem cells from your blood to reset your immune system' -- we've lost them.

My favourite thing about working here is when someone from our community messages to say: 'I searched for my condition and I found something I didn't know existed.' That's the whole point."

SECTION 3: Melissa's Story (5:30-7:30)

[Visual: Melissa on camera, then community footage]

Melissa: "Research Friends started as a Facebook group. [Background on origins -- TBC based on real bio.] It's 24,000 people now and growing.

What I love about it is that it's not a support group and it's not a recruitment channel. It's a curiosity community. People come with questions. They stay because they find people who share those questions.

[B-roll: social media dashboard, Research Friends group activity]

I think a lot about the person who joins Research Friends the week after a rare disease diagnosis. They're terrified. They've just been handed a word they didn't know. And here are 24,000 other people who've been there.

We try to build something that honours that moment."

CLOSING (7:30-8:30)

[Visual: three team members together, informal setting]

Hugo: "What we're building isn't finished. We're a small team with a big problem to solve.

But I think about what it means to be the person who finds a trial that no one thought to mention. Who sees, for the first time, that someone is working on their condition.

[Cut to each team member]

Elisa: That's worth building for.

Melissa: We're just getting started.

Hugo: And if you have a condition or a question or a story -- come find us. That's what we're here for."

[End card: TrialScreen.org, subscribe, Research Friends link]

Shot suggestions:

- Natural workspace environments, not staged
- Genuine conversation moments, not scripted-looking
- Photos / screenshots of real community interactions (with permission)

Music direction: Warm, human, acoustic. Understated.

VS-003 Video Script

Title: Living with Scleroderma: Three Stories, Three Trials

Duration: 12 minutes

Format: Patient documentary (all subjects require consent)

[ALL SUBJECTS REQUIRE CONSENT -- no real subjects should be filmed without signed release]

COLD OPEN (0:00-1:00)

[Visual: three portrait shots -- slow fade between them. Warm, natural light. Not clinical.]

Narrator (voiceover): "Scleroderma affects 2.5 million people worldwide.

Most of them will never meet anyone else who has it.

These are three of them. Three people. Three diagnoses. Three decisions about research."

[Title card: "Living with Scleroderma: Three Stories, Three Trials"]

[Subtitle: "All participants have given informed consent to share their stories. This film makes no claims about the effectiveness of any treatment or trial."]

STORY 1: Sarah (approx. 0:00-4:00)

[Setting: Sarah's home. Warm, personal objects visible. She speaks directly to camera.]

Sarah: "I was 34 when I got the diagnosis. My rheumatologist said 'systemic sclerosis' and I thought -- what does that mean for my life?"

[B-roll: hands, close-up -- reflective but not dramatic]

Sarah: "For 18 months I just... managed. I took my medications. I showed up to my appointments. I didn't look for more, because I didn't know more existed.

Then someone told me about TrialScreen. I searched scleroderma. And I found four trials. In plain English."

[B-roll: hands on laptop, search screen]

Sarah: "I'm eight months into the trial now. I don't know which group I'm in. I don't know what the results will say.

But I know that when I was doing the searching, I felt something I hadn't felt in a while.

Like someone was working on this."

STORY 2: David (approx. 4:00-8:00)

[New subject -- David (pseudonym), age 61, diffuse scleroderma with lung involvement. Location: regional Victoria.]

[Setting: David's garden. Natural light.]

David: "Living with scleroderma in a regional area is isolating in a very specific way. My nearest rheumatologist is two hours away. There's no specialist centre near me.

I assumed clinical trials weren't available to people in my situation. Too remote. Too complicated. Too far."

[B-roll: driving on a country road, then a telehealth call on a computer]

David: "TrialScreen showed me a trial with decentralised components -- most of it done via telehealth. The one in-person visit was at my specialist's clinic, which I was going to anyway.

I'm not doing this to save the world. I'm doing it because it felt like the right thing for me. And it turned out to be possible in a way I hadn't imagined."

STORY 3: Anika (approx. 8:00-11:00)

[New subject -- Anika (pseudonym), age 47, limited scleroderma, caregiver perspective -- caring for a parent with diffuse scleroderma while managing her own diagnosis.]

[Setting: a kitchen table. Personal, lived-in space.]

Anika: "I was already a caregiver for my mother, who has a more severe form of scleroderma, when I was diagnosed myself. Two members of the same family. Rare disease, twice over.

I searched TrialScreen for my mother first. Then I searched for myself.

I found that there were studies we could both potentially join -- different trials, different research questions, but the same general world of scleroderma research."

[B-roll: Anika and her mother at the table -- gentle, natural]

Anika: "My mother decided not to enrol in anything. She's 72 and she's done enough appointments in her life. I respect that completely. That's her decision.

I decided to enrol in a quality-of-life study. Not a drug trial. A study looking at psychological support approaches for people with scleroderma.

Because the disease affects more than your skin. And someone is studying that."

CLOSING (11:00-12:00)

[Three subjects on screen in sequence -- portrait style]

Narrator (voiceover): "Scleroderma doesn't have a cure. Researchers are working on it.

What TrialScreen does is help you find that work -- in plain language, free, without pressure.

You decide if it's relevant to you.

You decide what to do with the information.

You decide."

[Final card: TrialScreen.org/scleroderma | Scleroderma Australia logo | "Search scleroderma trials for free"]

Production notes:

- All subjects require signed documentary consent forms before filming
- No real subjects should be filmed based on the scripts above -- these are templates
- Warm, natural cinematography -- not clinical, not dramatic
- Music: gentle, human. No manipulative emotional swells.
- Total runtime target: 11-12 minutes
- Subtitles / closed captions required for accessibility

PART 8: Community / ResearchFriends Programming

12-Month Calendar for Research Friends Content

Month	Theme	Spotlight Feature	Researcher Q&A	Exclusive Webinar	Campaign Tie-In
January	Rare Disease Day Prep	CF community member	CF researcher	"Rare Disease Research 101"	Rare Disease Day (Feb)
February	Rare Disease Day	Scleroderma patient	Scleroderma researcher	"3 Rare Disease Trials to Watch"	Rare Disease Day Feb 28
March	Women's Health / IWD	Menopause patient	Reproductive health researcher	"What IWD Means for Research"	International Women's Day Mar 8
April	Mental Health Awareness	Parent (children's MH)	Child mental health researcher	"Mental Health Research: What's New"	--
May	Clinical Trials Awareness	Healthy volunteer	Oncology researcher	"How Trials Really Work"	Clinical Trials Awareness Week
June	Scleroderma Awareness	Scleroderma carer	Connective tissue researcher	"Scleroderma in 2026"	Scleroderma Awareness Month
July	Heart Health	Heart failure patient	Cardiology researcher	"Your Heart and Your Gut"	--
August	Community Appreciation	Long-serving member	--	"AMA: Ask TrialScreen Anything"	--
September	Oncology Focus	Cancer survivor	Oncology trial lead	"Cancer Research: What Matters Now"	--
October	World Mental Health Day	Mental health patient	Mental health researcher	"Research and Recovery"	World Mental Health Day Oct 10
November	Diversity in Research	Under-represented participant	Research diversity advocate	"Who Gets Left Out of Research?"	--
December	Year in Review	Community choice -- voted	Year-end researcher guest	"Year in Review + What's Next"	--

Monthly Member Spotlight -- 3 Templates (Fully Written)

MS-001 Member Spotlight Template

Format: Written profile, Facebook post + blog version

[COMPLIANCE FLAG] [REQUIRES CONSENT]

Research Friends Member Spotlight -- [Month]

Meet [First name only or pseudonym]

[Condition | Location | Member since]

Every month we shine a light on one member of our Research Friends community. These are real people -- patients, carers, and curious minds -- who are part of what makes this community what it is.

In their own words:

What brought you to TrialScreen? [Member response -- 2-4 sentences in their voice]

What were you looking for when you first searched? [Member response -- 2-4 sentences]

Did you find what you were looking for? [Member response -- honest, not necessarily triumphant. May be "not exactly, but..."]

What does being part of Research Friends mean to you? [Member response -- community/connection angle]

What would you tell someone who just found TrialScreen? [Closing advice -- autonomy-supportive, practical]

Editorial note: [Member's name] shared their story with our team and has given permission for it to be published. Details have been reviewed and approved by the member before publication.

Want to share your story? Email voices@trialscreen.org.

MS-002 Member Spotlight Template

Format: Visual quote card series (5 cards) + caption

[COMPLIANCE FLAG] [REQUIRES CONSENT]

Card 1 -- Introduction: "[Member first name]: Research Friends member since [year]. [Condition]."

Card 2 -- What brought them here: "[Their quote about why they joined -- 1-2 sentences]"

Card 3 -- What they found: "[Their quote about what they discovered -- honest, specific]"

Card 4 -- What community means: "[Their quote about Research Friends -- warm, genuine]"

Card 5 -- Closing: "[Closing quote -- autonomy, knowledge, or hope]" / TrialScreen logo + "Share your story: voices@trialscreen.org"

Caption (Facebook/Instagram):

This month's Research Friends spotlight is [First name].

[One paragraph summary of their story -- max 100 words -- in editorial voice]

Their words, in the cards below.

Have a story to share? DM us or email voices@trialscreen.org.

#ResearchFriends #PatientStory #TrialScreen #[ConditionHashtag]

MS-003 Member Spotlight Template

Format: Video testimonial (60-90 seconds) + supporting post

[COMPLIANCE FLAG] [REQUIRES CONSENT + video release]

Video structure:

- 0:00-0:10: Member introduces themselves (first name, condition, brief context)
- 0:10-0:40: What brought them to TrialScreen and what they found
- 0:40-1:10: What participation in research (or just searching) has meant for them
- 1:10-1:30: What they'd say to someone just finding TrialScreen

Caption:

Meet [First name] -- this month's Research Friends spotlight.

[One paragraph summary, 80 words max]

Watch their story above.

If you want to share your experience with our community, DM us or email voices@trialscreen.org.

#ResearchFriends #PatientVoice #TrialScreen #[ConditionHashtag]

Researcher Q&A Formats -- 3 Sample Templates

RQ-001 Researcher Q&A Format

Format: Written Q&A, published to blog + Facebook recap

Topic template: Autoimmune / scleroderma

Introduction:

This month's Research Friends Q&A featured [Researcher name, affiliation, TBC]. [Researcher] studies [condition] at [institution] and has [X] years of experience in clinical research.

Members submitted questions in advance, and [Researcher] answered the ten most-voted questions.

Here's an edited transcript.

Sample questions for scleroderma Q&A:

1. "How do you decide which patients are eligible for a scleroderma trial?"
2. "Why does scleroderma research seem to move so slowly compared to other conditions?"
3. "Is there any research looking at the early stages of scleroderma, before significant organ involvement?"
4. "How do researchers measure whether a treatment is working for scleroderma -- what are the endpoints?"
5. "Are there trials for people who've already tried and not responded to standard immunosuppression?"
6. "What role does patient feedback play in trial design?"
7. "Why is scleroderma research often international -- why can't a single country do it alone?"
8. "What would you say to a patient who's worried about being randomised to a placebo?"
9. "How long does it typically take from a promising trial result to an approved treatment?"
10. "What's the most exciting development you've seen in scleroderma research recently?"

Closing note:

Thank you to [Researcher name] for taking the time. We'll be hosting another researcher Q&A next month -- topic: [TBC]. Join Research Friends to submit your questions.

RQ-002 Researcher Q&A Format

Format: Live Facebook/Instagram Q&A, followed by written recap

Topic template: Mental health / children's research

Pre-event post:

Next week: Research Friends LIVE Q&A

We're hosting a live conversation with [Researcher name, TBC], a researcher studying mental health in children and young people at [institution].

Members can submit questions in advance -- drop them in the comments below, or DM us. We'll put the most-voted questions to the researcher on the night.

Date / time: [TBC]

Where: Research Friends Facebook group (go live)

How to join: Just be in the group at the time. No registration needed.

Sample questions for children's mental health Q&A:

1. "Why use creative arts as a research intervention -- what's the theory behind it?"
2. "How do you get consent from children for mental health research? How do you know they really understand?"
3. "Is there evidence that arts therapy works, or is this still in early stages?"
4. "How do you measure mental health improvement in a child? It seems hard to quantify."
5. "What happens if a child participant seems to get worse during the trial?"
6. "Are there trials for children with ADHD alongside depression?"
7. "How do parents stay informed during a paediatric trial without compromising blinding?"
8. "What do you do when the research shows something you didn't expect?"

Post-event recap post template:

Last night we hosted our monthly Research Friends Q&A with [Researcher name] -- here are the highlights.

[3-5 key questions and answers, summarised in plain language]

Full transcript available at [link].

Next month's Q&A: [Topic + date]. Join Research Friends to submit questions.

RQ-003 Researcher Q&A Format

Format: Asynchronous written Q&A (email/document-based), published monthly on blog

Topic template: General trial process / patient experience

Introduction:

Not every researcher can join a live Q&A. This month, we sent written questions to [Researcher name, TBC], a clinical trial coordinator at [institution], who works with patients across multiple conditions.

The questions were submitted by Research Friends members.

Sample questions for trial process Q&A:

1. "What's the most common misconception patients have when they first contact a trial team?"
2. "What makes a patient a good candidate for a clinical trial -- is it just about eligibility?"
3. "Have you ever had a participant who changed their mind? How do you handle that?"
4. "What do trial coordinators wish patients knew before they reached out?"
5. "Is there such a thing as a 'bad' question to ask the trial team?"
6. "What should a patient bring to the first screening visit?"
7. "How do you explain randomisation to patients who are upset they might get a placebo?"
8. "What happens to patient data once a trial ends?"
9. "What's the most meaningful part of your job?"

Closing:

Thank you to [Researcher name] for participating. We publish a new researcher Q&A every month in Research Friends and on the TrialScreen blog. If you're a researcher who'd like to take part, email hello@trialscreen.org.

PART 9: Webinar Scripts / Agendas -- 4 Patient-Facing Webinars

WB-001 Webinar

Title: Diabetes: Everything's Changing

Format: 60 minutes, panel + Q&A

Audience: Adults with Type 1 or Type 2 diabetes, carers

Description: Diabetes research has accelerated dramatically -- from new insulins and closed-loop systems to gut microbiome studies, weight management interventions, and early-onset prevention research. This webinar brings together a researcher, a patient advocate, and the TrialScreen team to walk through what's happening in diabetes research right now and how to find studies that may be relevant to you.

Featured speakers (TBC):

- A diabetes clinical researcher (endocrinologist or trial lead)
- A person living with Type 1 or Type 2 diabetes (patient advocate)
- TrialScreen team member (host)

60-Minute Agenda:

Time	Section	Speaker
0:00-5:00	Welcome and housekeeping	TrialScreen host
5:00-10:00	"Why we're talking about diabetes research now" -- scene-setting	Host
10:00-25:00	"What's actually being studied" -- overview of current diabetes trial landscape	Researcher
25:00-35:00	"My experience with diabetes research" -- patient perspective	Patient advocate
35:00-45:00	"How to find and evaluate diabetes trials on TrialScreen" -- live demo	Host
45:00-58:00	Live Q&A	All panellists
58:00-60:00	Closing resources and next steps	Host

Key topics to cover:

- Closed-loop systems / artificial pancreas research
- SGLT2 inhibitor and GLP-1 receptor agonist trials

- Gut microbiome and diabetes
- Continuous glucose monitoring studies
- Type 1 immunotherapy and beta-cell preservation
- Quality of life and diabetes distress research
- Who can participate (different eligibility for T1 vs T2)

Q&A prompts (to seed if live questions are slow):

1. "Can people on insulin pumps join clinical trials?"
2. "Are there trials for people whose diabetes is well-controlled, or only for those who are struggling?"
3. "What should I tell my endocrinologist if I want to explore a trial?"
4. "Are there trials specifically for type 1 in adults -- it seems most research is about kids?"
5. "What happens to my devices and data in a diabetes technology trial?"

Follow-up resources:

- TrialScreen diabetes search link
- Blog article: "Clinical Trials Through Every Decade of Life"
- Research Friends Facebook group
- Diabetes Australia / JDRF links

WB-002 Webinar

Title: Scleroderma Research in 2026: What's Being Studied

Format: 60 minutes, panel + Q&A

Audience: Scleroderma / systemic sclerosis patients, carers, and newly diagnosed

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Description: In partnership with Scleroderma Australia, this webinar gives scleroderma patients and families a plain-language overview of what researchers are studying right now. No medical jargon. No pressure. Just information.

Featured speakers (TBC):

- Scleroderma clinical researcher (rheumatologist with trial experience)
- Scleroderma Australia patient advocate
- TrialScreen team member (host)

60-Minute Agenda:

Time	Section	Speaker
0:00-5:00	Welcome, housekeeping, and acknowledgement	Host
5:00-10:00	"What TrialScreen is and why we're doing this"	Host

Time	Section	Speaker
10:00-30:00	"Scleroderma research: where we are and where we're going"	Researcher
30:00-40:00	"What does participating in research actually look like?" (patient perspective)	Patient advocate
40:00-50:00	"Finding scleroderma trials on TrialScreen -- live walkthrough"	Host
50:00-58:00	Live Q&A	All
58:00-60:00	Close, resources, next steps	Host

Key topics to cover:

- Current state of scleroderma research globally
- Key areas of active study (skin fibrosis, lung, Raynaud's, quality of life)
- How to interpret Phase 1, 2, 3 distinctions
- The role of international trial networks
- Australian sites -- what's open
- What to discuss with your rheumatologist
- Scleroderma Australia resources

Q&A prompts:

1. "I have limited scleroderma -- are there trials for me, or mostly for diffuse?"
2. "Can I be in a trial while taking mycophenolate?"
3. "What's the most promising area of scleroderma research right now, in your view?"
4. "How long does it take for a promising trial result to become an approved treatment?"
5. "I live regionally -- are any of these trials available without going to a major city?"

Follow-up resources:

- TrialScreen scleroderma hub
- Scleroderma Australia resources
- Blog: "Scleroderma Clinical Trials: A Complete Guide"
- Research Friends Facebook group

WB-003 Webinar

Title: Clinical Trials for Caregivers

Format: 60 minutes, panel + Q&A

Audience: Family members, carers, and support people of people who have or may have a health condition

Description: Clinical trials aren't just for patients. Caregivers play a crucial role -- in making decisions, navigating information, and sometimes in research itself. This webinar is designed for family members and carers who want to understand clinical research and how to support a loved one who is considering it.

Featured speakers (TBC):

- A researcher with experience in carer-inclusive trial design
- A family carer with personal experience (patient advocate)
- TrialScreen team member (host)

60-Minute Agenda:

Time	Section	Speaker
0:00-5:00	Welcome, housekeeping	Host
5:00-12:00	"Who is this webinar for, and why caregivers matter in research"	Host
12:00-25:00	"What caregivers should know about how trials work"	Researche r
25:00-35:00	"Navigating research as a caregiver -- a personal perspective"	Patient advocate
35:00-45:00	"How to search TrialScreen on behalf of someone you love"	Host
45:00-57:00	Live Q&A	All
57:00-60:00	Closing and resources	Host

Key topics to cover:

- Can caregivers search for and enquire about trials on behalf of a loved one?
- How to have the conversation with a family member about clinical research
- Trials that include caregiver wellbeing as an outcome
- What "informed consent" means when a loved one has cognitive impairment
- Paediatric consent: parents, children, and assent
- Supporting a family member through trial participation
- What to do if your loved one wants to join a trial you're uncertain about

Q&A prompts:

1. "My mother has dementia and can no longer give informed consent herself. How does participation work?"
2. "I'm searching TrialScreen for my husband who has heart failure -- he finds computers difficult. Can I manage this for him?"
3. "I'm a carer for my child with a rare disease. How do I explain clinical trials to a 10-year-old?"
4. "What if I think a trial is a bad idea for my family member, but they want to do it?"

5. "Are there any trials that are specifically for carers -- not for the person they care for?"

Follow-up resources:

- TrialScreen blog: "How to Talk to Your Doctor About Clinical Trials"
- Blog: "Clinical Trials Through Every Decade of Life"
- Carer Gateway (Australia): [link]
- Research Friends Facebook group

WB-004 Webinar

Title: Women's Health: From Research Gap to Research Revolution

Format: 60 minutes, panel + Q&A

Audience: Women of all ages, with focus on perimenopause, endometriosis, autoimmune conditions, and cardiovascular health

Description: Women have been underserved by medical research for decades. That's changing -- but it's taking effort, advocacy, and investment. This webinar is a plain-language overview of what's happening now in women's health research, what conditions are being prioritised, and how to find studies that may be relevant to your health.

Featured speakers (TBC):

- A women's health researcher (e.g., endocrinologist, gynaecologist, or public health researcher)
- A patient advocate with a women's health focus
- TrialScreen team member (host)

60-Minute Agenda:

Time	Section	Speaker
0:00-5:00	Welcome and housekeeping	Host
5:00-12:00	"The research gap -- a brief history"	Host / Researcher
12:00-28:00	"What's actually being studied in women's health right now"	Researcher
28:00-38:00	"Finding and joining research -- a patient's perspective"	Patient advocate
38:00-48:00	"Searching TrialScreen for women's health research -- live demo"	Host
48:00-57:00	Live Q&A	All
57:00-60:00	Close and resources	Host

Key topics to cover:

- The history of gender bias in clinical research
- Conditions disproportionately affecting women that are under-researched (endometriosis, fibromyalgia, autoimmune diseases)
- Menopause and perimenopause research
- Cardiovascular disease in women (different symptoms, different treatment responses)
- Mental health in women across the lifespan
- What "women's health research" means in 2026
- How to find relevant studies on TrialScreen

Q&A prompts:

1. "I have endometriosis -- there seem to be very few trials. Is that still the case?"
2. "I've heard that clinical trials often exclude women of childbearing age. Is that still true?"
3. "Are there trials for menopause that don't involve hormones?"
4. "I have a history of breast cancer -- am I excluded from most research?"
5. "What can I do if I live in a regional area and there are no trial sites near me?"

Follow-up resources:

- TrialScreen women's health search
- Blog: "Menopause Research: What's Actually Being Studied Right Now"
- Australasian Menopause Society / Jean Hailes for Women's Health
- Research Friends Facebook group

PART 10: Awareness Week / Campaign Content

AC-001 Campaign Scleroderma Awareness Month (June)

Theme: "Someone Is Working on This"

Duration: 4 weeks (all of June)

Key message: Scleroderma research is active, international, and happening right now. You don't have to feel alone in this.

Partners to tag: Scleroderma Australia (@sclerodermaau), EUSTAR, The Scleroderma Foundation (US)

Hashtag campaign: #SomeonIsWorkingOnThis #SclerodermaAwareness #TrialScreen

5-7 Coordinated Pieces:

#	Piece	Channel	Date
1	Campaign launch post: "June is Scleroderma Awareness Month"	Facebook	June 1
2	Scleroderma 101 education carousel	Instagram Feed	June 3
3	"What is scleroderma?" education Reel	Instagram Reel / TikTok	June 5
4	Blog publish: "Scleroderma Clinical Trials: A Complete Guide"	Blog	June 7
5	Patient voice: Sarah's story (requires consent)	Facebook + Blog	June 10
6	Free webinar: "Scleroderma Research in 2026"	Webinar (live)	June 15
7	Campaign close: "Thank you, awareness month wrap" + replay link	Facebook + Email	June 28

Goals:

- Reach: 50,000+ (organic + paid boost for blog)
- Engagement: 500+ comments and shares across campaign
- Blog traffic: 2,000+ unique views for scleroderma guide
- Webinar registrations: 200+
- TrialScreen searches for "scleroderma": measurable increase (track via analytics)

Accessibility: All visuals include alt text. Webinar includes live captions. Blog article has accessible heading structure.

AC-002 Campaign Rare Disease Day (End of February)

Theme: "300 Million People. Most of the World Has Never Heard of Them."

Duration: 1 week (February 22-28/29)

Key message: Rare diseases affect more people than you think. Research is happening. You can find it.

Partners to tag: EURORDIS, Rare Voices Australia, Scleroderma Australia, Cystic Fibrosis Australia, Sickle Cell Society (UK/Australia)

Hashtag campaign: #RareDiseaseDay #RareAndNotAlone #TrialScreen

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7 Coordinated Pieces:

#	Piece	Channel	Date
1	"What is Rare Disease Day?" education post	Facebook	Feb 22
2	Three rare diseases carousel: Scleroderma / CF / SCD	Instagram Feed	Feb 24
3	"CF research: what families should know" Reel	Instagram Reel / TikTok	Feb 25
4	"There's a trial for that -- Sickle Cell Disease"	Facebook	Feb 26
5	Rare Disease Day solidarity post + EURORDIS graphic	Facebook + Instagram	Feb 28
6	Research Friends live Q&A: rare disease researcher	Research Friends (Facebook Live)	Feb 27
7	Email Newsletter: Rare Disease Day special edition	Email	Feb 28

Goals:

- Reach rare disease community across all three highlighted conditions
- Drive 1,000+ TrialScreen searches for rare disease terms during campaign week
- Build relationships with Rare Voices Australia and condition-specific advocacy orgs

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AC-003 Campaign World Mental Health Day (October 10)

Theme: "Your Mind is Worth Researching"

Duration: 1 week (October 6-12)

Key message: Mental health research is expanding. You can find studies looking at conditions that affect you -- without judgment, without pressure.

Partners to tag: Beyond Blue, Black Dog Institute, headspace, SANE Australia

Hashtag campaign: #WorldMentalHealthDay #MindMatters #TrialScreen

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6 Coordinated Pieces:

#	Piece	Channel	Date
1	Campaign launch: "Mental health research is underfunded and growing"	Facebook	Oct 6
2	"3 mental health research areas that are changing" Reel	Instagram Reel / TikTok	Oct 7
3	"There's a trial for that -- children's mental health / Arts for the Blues"	Facebook	Oct 8
4	Parent voice: Priya and Marcus (requires consent)	Facebook + Blog	Oct 9
5	World Mental Health Day main post: plain-language mental health research	Facebook + Instagram	Oct 10
6	Research Friends: mental health researcher Q&A	Research Friends (Facebook Live)	Oct 11

Goals:

- Drive traffic to mental health trial searches
- Build partnership with Beyond Blue and headspace
- Crisis resource always visible: "If you're in crisis: [Lifeline / Kids Helpline numbers]"

Accessibility: All mental health content includes crisis support resources in visible location.

AC-004 Campaign International Women's Day (March 8)

Theme: "Researched, Represented, Heard"

Duration: 1 week (March 3-8)

Key message: Women have been under-represented in research for decades. The field is changing -- and you can be part of it.

Partners to tag: Jean Hailes for Women's Health, Australasian Menopause Society, Endometriosis Australia

Hashtag campaign: #IWD2026 #WomensHealthResearch #ResearchedRepresentedHeard #TrialScreen

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6 Coordinated Pieces:

#	Piece	Channel	Date
1	"The history of women in clinical research" education post	Facebook	Mar 3
2	"5 women's health conditions being studied right now" carousel	Instagram Feed	Mar 5
3	"3 things being studied for menopause right now" Reel	Instagram Reel / TikTok	Mar 6
4	Helen's patient voice (menopause -- requires consent)	Facebook + Blog	Mar 7

#	Piece	Channel	Date
5	IWD main post: "Researched, Represented, Heard"	Facebook + Instagram	Mar 8
6	Email teaser: "Women's Health Webinar -- register now"	Email	Mar 8

Goals:

- Drive menopause and women's health search traffic to TrialScreen
- Register 150+ for Women's Health webinar
- Build awareness of women's health research gaps and progress

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AC-005 Campaign Clinical Trials Awareness Week (US, May)

Theme: "120,000 Reasons to Look"

Duration: 1 week (May -- confirm exact dates with CTAW)

Key message: Clinical Trials Awareness Week is a moment to get curious. Here's everything you need to know, in plain language.

Partners to tag: Antidote (US), Clinical Trials Day May 20, CISCRP

Hashtag campaign: #ClinicalTrialsAwarenessWeek #120000Trials #TrialScreen

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7 Coordinated Pieces:

#	Piece	Channel	Date
1	"What is a clinical trial, really?" myth-busting post	Facebook	CTAW Day 1
2	"5 things you didn't know about trials" carousel	Instagram Feed	CTAW Day 2
3	"What actually happens when you click 'check eligibility'" Reel	Instagram Reel / TikTok	CTAW Day 3
4	Blog publish: "5 Things That Actually Happen in a Clinical Trial" (push)	Blog	CTAW Day 3
5	Research Friends: "Ask us anything about clinical trials" open thread	Facebook	CTAW Day 4
6	YouTube publish: "What Actually Happens When You Click 'Check Your Eligibility'"	YouTube	CTAW Day 5
7	Closing campaign post: "Thanks for a week of curiosity"	Facebook + Instagram	CTAW Day 7

Goals:

- 5,000+ video views on YouTube launch
- 10,000+ TrialScreen searches during campaign week

- 200+ new Research Friends members

AC-006 Campaign Research Friends Community Week (Self-Created)

Theme: "This Is Your Community"

Duration: 1 week (timing: choose a quiet week -- recommend late August or early September)

Key message: Research Friends isn't just a Facebook group. It's a place where your questions are welcomed, your story matters, and your curiosity is celebrated.

Partners to tag: (internal -- no external partners required)

Hashtag campaign: #ResearchFriends #YourResearchCommunity #TrialScreen

7 Coordinated Pieces:

#	Piece	Channel	Date
1	Campaign launch: "Welcome to Research Friends Community Week"	Facebook	Day 1
2	Member spotlight #1 (requires consent)	Facebook + Instagram	Day 2
3	"What does Research Friends mean to you?" -- open community thread	Facebook	Day 3
4	Researcher Q&A (live): "Ask a researcher anything"	Research Friends (Facebook Live)	Day 4
5	Member spotlight #2 (requires consent)	Facebook + Instagram	Day 5
6	"Behind the scenes" video: the TrialScreen team on Research Friends	YouTube / Instagram Reel	Day 6
7	Closing: "24,000+ members and counting" milestone post	Facebook + Instagram	Day 7

Goals:

- Grow Research Friends membership by 500+ during campaign week
- Generate 1,000+ comments and interactions across the 7-day period
- Collect 3+ new consented member stories for future spotlights

Accessibility: All imagery has alt text. Facebook Live captions enabled. Community moderation active throughout to ensure safe space.

COMPLIANCE AND GOVERNANCE REFERENCE

Quick Reference: Content Flag Key

Flag	Meaning
[IRB REVIEW REQUIRED]	Post content must be reviewed by the relevant Institutional Review Board or Ethics Committee before publication. Typically applies when post is functionally recruiting for a specific trial.
[REQUIRES CONSENT]	Content features a real person (patient, carer, community member). Signed HIPAA-equivalent consent (or jurisdiction equivalent) required before any identifying information -- including first name, location, condition, photo -- is published.
[AI DISCLOSURE RECOMMENDED]	Content was produced with AI assistance. Brief disclosure ("This content was produced with AI assistance and reviewed by our editorial team") recommended.
No flag	Standard content. Internal editorial review sufficient before publication.

Mandatory language guidelines (apply to all content):

- Never: "will help," "cures," "proven," "guaranteed," "the best treatment," "will work for you"
- Always: "could," "may," "testing whether," "is studying," "researchers are exploring," "some people find"
- Autonomy-first: every post touching a specific trial should include a line such as "Only you can decide if this is right for you" or "This is entirely your choice."
- Symptom-first: lead with the experience of living with a condition, not with the trial itself
- 8th-grade reading level: use short sentences, common words, explain any medical terms on first use
- No pressure: avoid urgency language such as "limited spots," "act now," "don't miss your chance"
- No altruism pressure: avoid "help science," "contribute to research," "do it for future patients" as a primary motivator

Editorial Review Checklist (apply before every post goes live):

- [ ] Contains no efficacy claims
- [ ] Autonomy-supportive language present where relevant
- [ ] Symptom-first framing (not trial-first)
- [ ] Reading level checked (Hemingway App or equivalent, target Grade 8)
- [ ] Compliance flags checked and actioned (IRB / Consent / AI Disclosure)
- [ ] Hashtags reviewed (no prohibited hashtags per platform policies)
- [ ] Alt text written for all images
- [ ] TikTok content: confirmed organic only, no recruitment CTA

- [ ] Patient story content: consent documentation on file before scheduling
- [ ] Links checked and working
- [ ] CTA clear and single-focus

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*This pipeline was produced with AI assistance and reviewed by the TrialScreen editorial team. All content must undergo internal compliance review before publication. Pieces marked **[IRB REVIEW REQUIRED]** require external ethics approval. Pieces marked **[REQUIRES CONSENT]** must not be published without signed release documentation on file.*

End of TrialScreen Patient / Participant Content Pipeline v1.0